

OPPORTUNITY REVIEW

DIRECT-TO- EMPLOYER GTM STRATEGY

Exploring a MU Healthcare DTE Health Plan





Our Founding and Heritage: 46 Years of Striving for Honesty in Healthcare

Founded Key Benefit Administrators in 1979

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator focused on honest healthcare delivery.

Sold to Blue Cross/Blue Shield in 1985

Strategic partnership with BCBS of Indiana (then part of the Associated Group) provided capital and market access while maintaining operational independence.

Reacquired KBA in 1992

Larry bought the business back from BCBS in 1992, returning it to complete independence and patient-focused mission alignment.

Built the Largest Independent TPA in the U.S.

Under his leadership since the buyback, KBA has become the country's largest independently owned third-party administrator, managing millions of members across diverse markets.

Never Forgotten Who Gives Healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

Key Benefit Administrators



DISCUSSION OPTIONS

1. Why KBA?
2. Why MU Health Care?
3. Financial Assumptions and Projections
4. Implementation and Operations
5. Network Management

WHY KBA?





- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
 - 3,000+ Corporate Clients
 - 3 Million Members under Management
 - 700+ Employees



MARKET LEADER IN PRIVATE LABELED DTE PLANS



PUTTING A NAME TO THE PROBLEM

CHANNEL DEPENDENCY RISK



PORTER'S FIVE FORCES

Channel dependency risk is a form of bargaining power of suppliers.

EXAMPLES OF CHANNEL DEPENDENCY RISK

Amazon hurts publishing houses

App stores hurt mobile apps

Gig economy platforms hurt gig workers

Netflix hurts film studios

DIRECT-TO-EMPLOYER (DTE)

Your Health Plan
Focused on a Direct
Relationship with Self-
Funded Employers in
Your Market



YOU Pick the Name of the Health Plan



YOU Determine the Fees Charged



YOU Practice the Medicine



YOU Receive the Profits



WE Make it Successful

DTE KEYS TO SUCCESS FOR MU HEALTHCARE

1

DTE Plan Sells

DTE plans only work when the network is built around a strong brand with good access and competitive costs.

MU Healthcare is the quality and access leader in this service area (see slide 8). Market size and brand quality are likely to negatively impact plan growth

2

Estimates for Margin and Steerage Hold

KBA models high, medium and low estimates for net income based on (1) estimates of commercial margin and (2) the within-DTE-network steerage rate.

Profit growth will hold if these assumptions accurately play out as expected for MU Healthcare.



Our 'Why': The Photo Moment

THE KBA REWARD THROUGH SEEING A DIFFERENCE

Better Care, Lower Costs, Real Partnerships

The ultimate measure of our success is the moment when all stakeholders gather to review real progress - improved population health and dramatically lower healthcare spending. This is where theory becomes reality, and everyone wins.



All Stakeholders at the Table

Employer, Provider, KBA, and Broker/Consultant
Broker/Consultant working in unified
partnership toward common goals



Reviewing Real Progress

Measurable improvements in population health
health outcomes combined with verifiable
verifiable reductions in total healthcare
spending



Deep Loyalty Built on Value

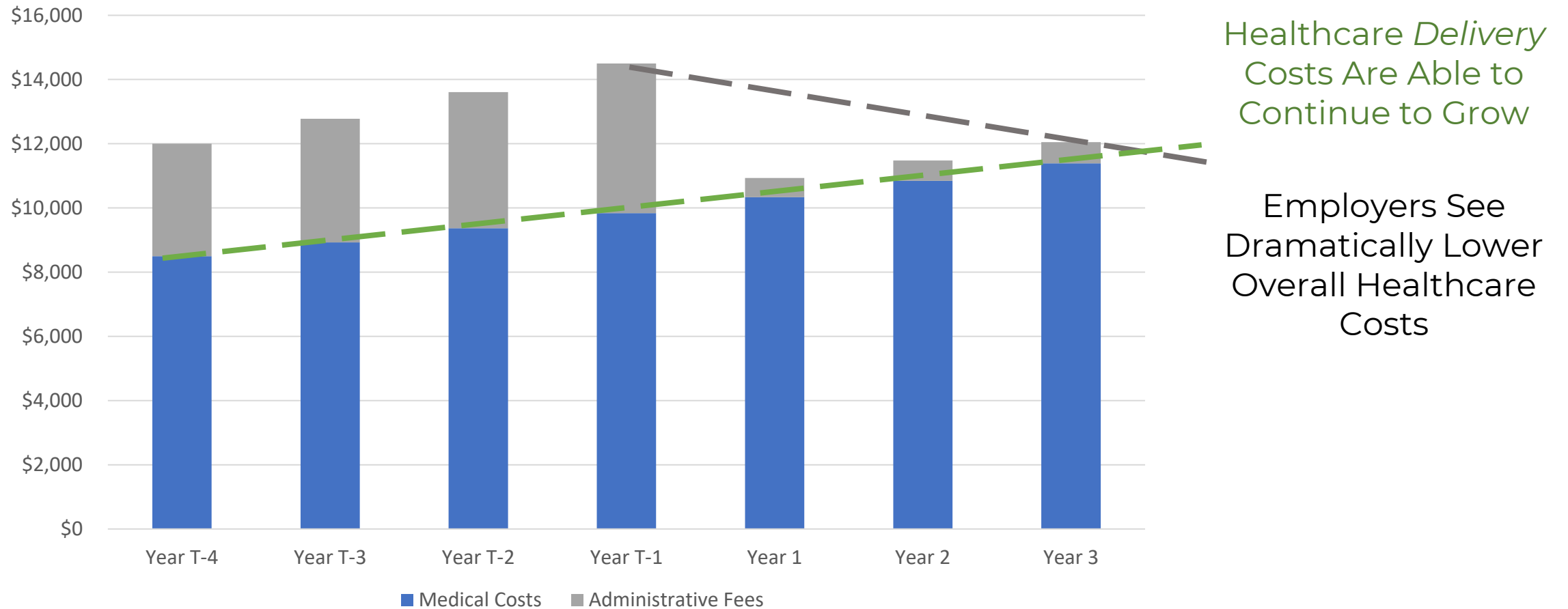
Authentic relationships between employer and
employer and provider organization based on
based on demonstrated value delivery, not
not contracts



Direct-to-Employer Strategy: POWERFUL EMPLOYER VALUE

- Reduced medical spend through lower unit costs and better integrated care
- “At the table” with MU Healthcare
- Aligned financial incentives
- Complete transparency for unlimited visibility into health benefit plan expenses and cost drivers
- Aggressive stop-loss premiums and policy terms
- Increased employee satisfaction
- Improved cash flow & stabilization of future medical cost trend

THE WIN/WIN OF CUTTING OUT THE MIDDLEMAN



Proof: 98% of Our Clients Beat Healthcare Inflation

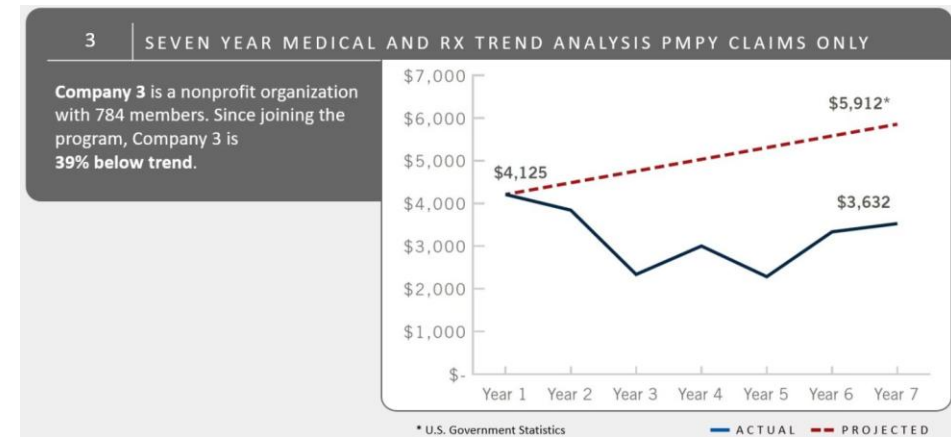
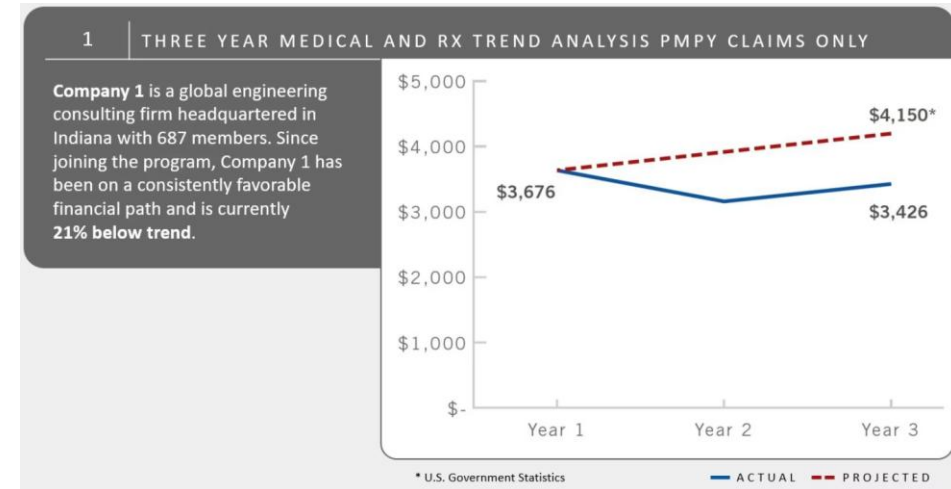
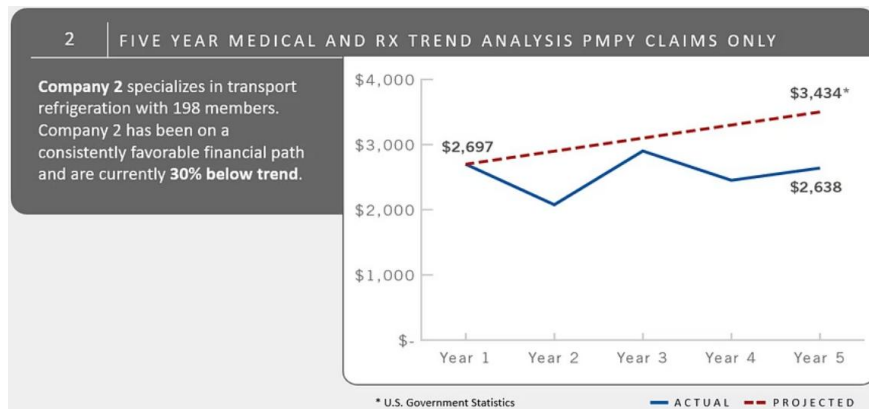
The Red Line = Healthcare CPI

The year-over-year healthcare component of the consumer price index represents typical market healthcare cost increases that employers face nationwide.

The Blue Line = Main Street Management

What happens when you manage your health on Main Street with direct provider relationships, transparent pricing, and aligned incentives.

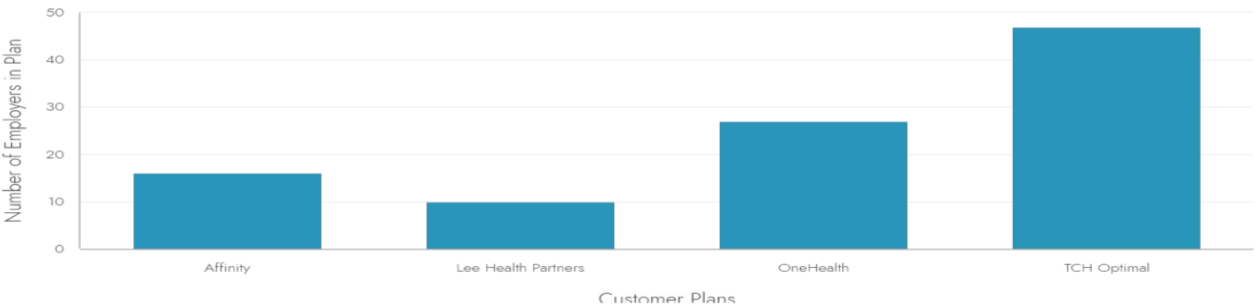
For our clients, 98% of our Blue Lines are BELOW their Red Lines!



This consistent outperformance demonstrates that direct-to-employer partnerships deliver measurable, sustainable value for all stakeholders involved.

KBA Plan Growth in the First Two Years

An average of 25 employer groups in plan within the first two years!



1,600

Louisville KY

OneHealth Plan achieved strong initial growth

1,800

Bentonville AR

Northwest Health Plan exceeded projections

6,400

Cincinnati OH

TCH Optimal demonstrated exceptional market acceptance

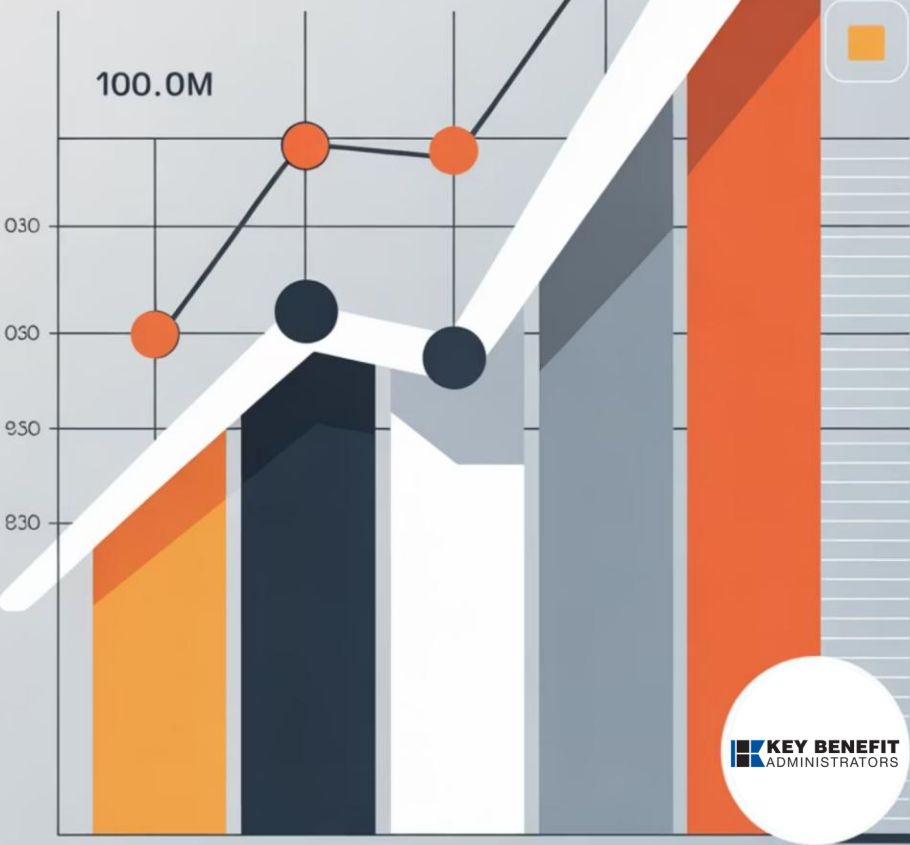
12,900

Fort Myers FL

Lee Health Care Partners achieved remarkable growth

These results demonstrate KBA's proven ability to build sustainable membership growth across diverse market conditions and geographic conditions and geographic regions. Each partnership reflects our commitment to long-term success rather than short-term enrollment term enrollment spikes.

Membership Growth



COMMUNITY BASED SALES STRATEGY



Locally based sales rep with strong community ties



Collaboration with DTE provider partners to identify strategic employers for early adoption and sales



Work with select broker distribution partners with aligned incentives and goals



Pipeline reporting and collaboration with our DTE provider partners



Marketing and promotion of the new option in the community

COMPLETE MARKETING SYSTEM

- Customized Branding and Messaging
- Customized Brochures
- Customized Web Site
- Targeted Employer Outreach
- Sales System (Social Media, Email, Text, etc.)



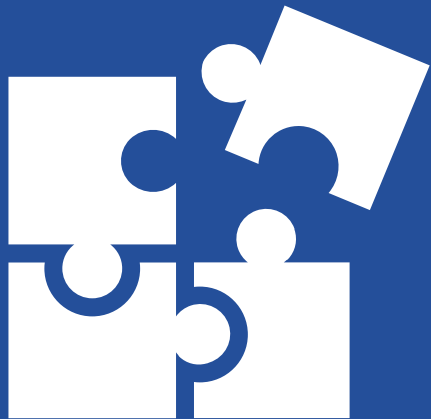
INTEGRATION WITH EPIC EHR SYSTEM

**KBA shares
evidence-based
standards of care for
27 chronic conditions
directly into Epic**



Hyperthyroidism: Yearly Clinical Evaluation, TSH and T4

THE MISSING PIECE TO VALUE- BASED CARE



- Patient engagement is the weak link in delivering value-based care; MU Healthcare's clinical leadership and employer participation can drive better outcomes.
- KBA's proactive care-management team supplements your care-management efforts in outreach to clients.
- MU Healthcare excels in Standards of Care completion rates for the Chronically Ill, improving care and costs.
- Integrating MU Healthcare's managed care with KBA's programs positions MU Healthcare as the value leader.

TRANSPARENT PBM INTEGRATION

- Support of MU Healthcare's retail pharmacy(s)
- Optimize MU Healthcare's 340B drug pricing program
- Allows MU Healthcare to create its own PBM program if desired
- Custom telephone number with personalized scripting and separate call tracking & reporting
- Fully customizable member welcome booklet & materials
- White labeled formulary document and marketing materials

CUSTOM CLIENT SUPPORT:

- Specialized call center capabilities
- Fully customizable plan designs
- Operational access to the platform
- Customized process and functions to promote DTE strategy

CLINICAL FLEXIBILITY:

- Full portfolio of custom formulary offerings – with optimized formulary shield
- Clinical programs that drive towards overall DTE strategy
- Options for additional drug savings by driving members to lower cost alternatives
- Flexibility to customize or layer on utilization management programs

STOP-LOSS INSURANCE

THE KBA EXPERIENCE

- Deep relationships with 20+ stop loss underwriters and carriers
- Over \$150M in annual stop loss premium placement
- Comprehensive pricing and medical service analysis performed for each DTE health plan to ensure proper pricing
- KBA provides the administration required of the stop-loss markets:
 - Bordereau Reporting
 - Claim Reporting and Reconciliation
 - Evaluation of Networks
 - Care Management
 - Initial Underwriting
 - Negotiation of rates and terms
 - Plan Design Actuarial

Tokio Marine

Nationwide

Berkshire

Hathaway

Sun Life

Liberty Mutual

Granular (Google)

Companion Life

Crum & Forster

American Fidelity

RGA

DTE PROGRAM MARKETING & SALES

KBA WORKS WITH YOU TO MANAGE & SELL:

- Branding & Messaging
- Direct & Brokerage Sales Distribution – Focused Sales Representative
- Brochures, Web Site, Media, etc.
- Stop-Loss/Level Funded Down to 20 EE Lives
- Sell Through Established Brokers in Your Service Area



36.5% OF AMERICANS DELAYING MEDICAL CARE DUE TO COST

The high cost of healthcare in the United States is a significant barrier to accessing necessary medical services. According to recent data, over a third of Americans have had to skip or delay their medical care in the past year due to the financial burden. This is a concerning trend that highlights the need for more affordable and accessible healthcare options.

THE KBA HEALTHCARE CONVENIENCE CARD

Creating EPO Steerage in a PPO Network

The Healthcare Convenience Card is eligible to be used only for specific merchant codes.

1. To utilize at Specific Healthcare Providers
 - a. Designated Hospitals
 - b. Urgent Care Facilities
 - c. Physicians
 - d. Eligible Ancillary Providers
2. Out of Pocket Expenses Qualify
3. 0% Interest Rate
4. Employee Determines Repayment Schedules
5. To Qualify: Answer One Simple Question



The Key Incentive Plan

Base Medical + Preferred Plus Benefits = Your Deductible Can Simply Vanish

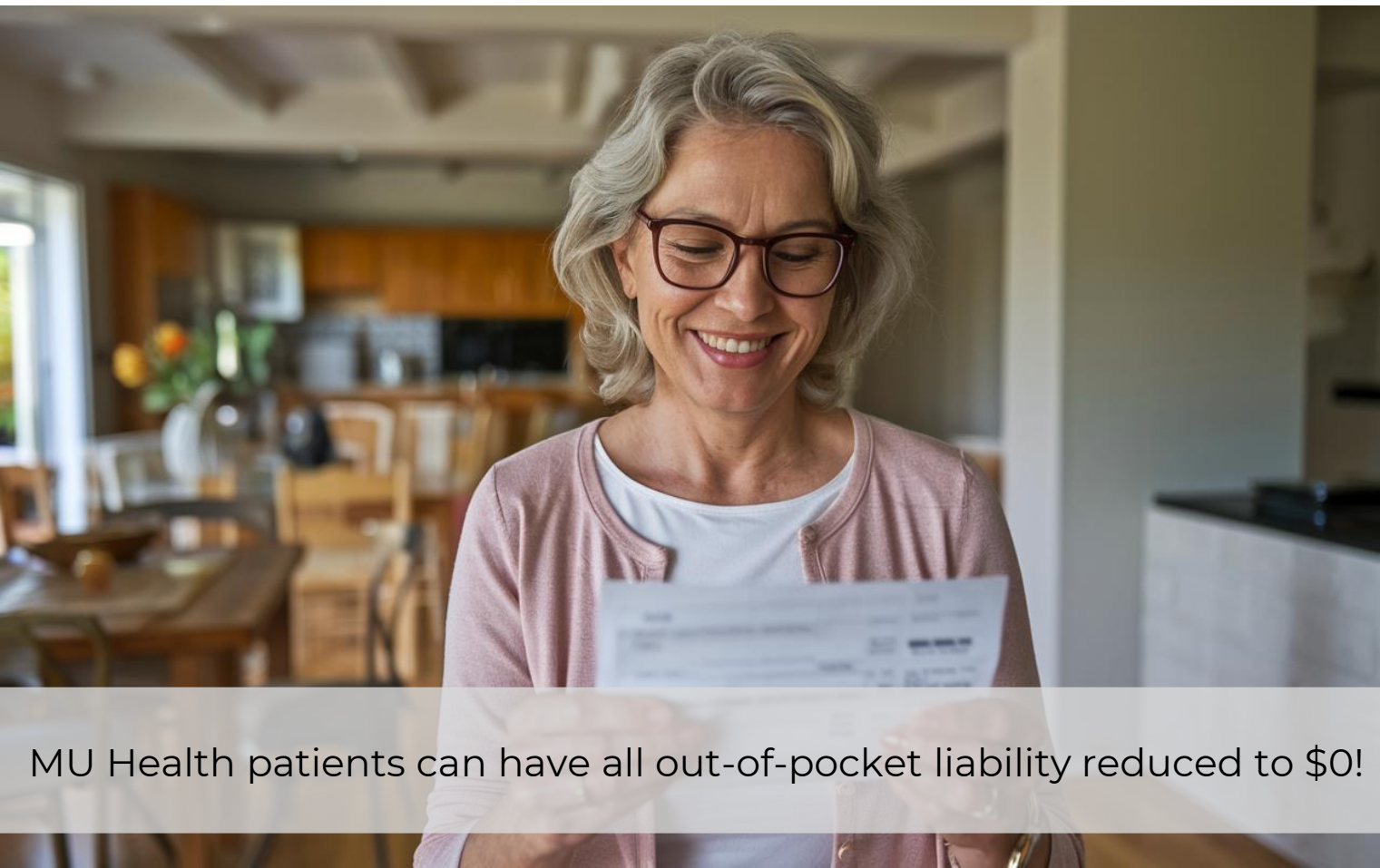
Example: Member with a \$2,500 deductible / \$4,500 max out-of-pocket has a heart attack and cardiac surgery, resulting in a 5-day in-patient stay. In this example, the member would have a \$4,500 OOP with \$5,050 Preferred Plus Benefits for a **net OOP \$0 due to the preferred provider versus \$4,500 OOP exposure at any non-preferred provider.**

BASE MEDICAL BENEFITS*		PREFERRED PLUS BENEFITS		
Deductible		Benefits automatically included in your plan!		
Single	\$2,500		Preferred Provider	Non-Preferred Provider
Family	\$4,500	OP Radiology/X-Ray	\$200	\$0
Coinsurance		OP Diagnostic Labs	\$100	\$0
80% / 20%		Preferred Hospital Stay (\$500/day, 5 days)	\$2,500	\$0
Maximum Out-of-Pocket		Critical Illness Benefit	\$2,000	\$0
Single	\$4,500	Health Screening Benefit	\$125	\$0
Family	\$9,000	Total Employee OOP	\$4,500	\$4,500
Chronic Disease Management Regimens of Care Covered at 100%		Preferred Plus Benefits	\$4,925	\$0
Healthcare Convenience Card to Cover Out-of-Pocket Expenses at 0% interest		Employee Amount Due to Provider	\$0	\$4,500
Health Guardian Account to help members manage their healthcare expenses at Preferred Providers		Additional Benefit Payable to the Health Guardian Account	\$425	\$0

*Base Medical Benefits are for illustration purposes only. Base benefits can be customized to meet client needs.

The Benefit Steering the Patient to MU Health

What Will this Claim Cost Me?



MU Health patients can have all out-of-pocket liability reduced to \$0!

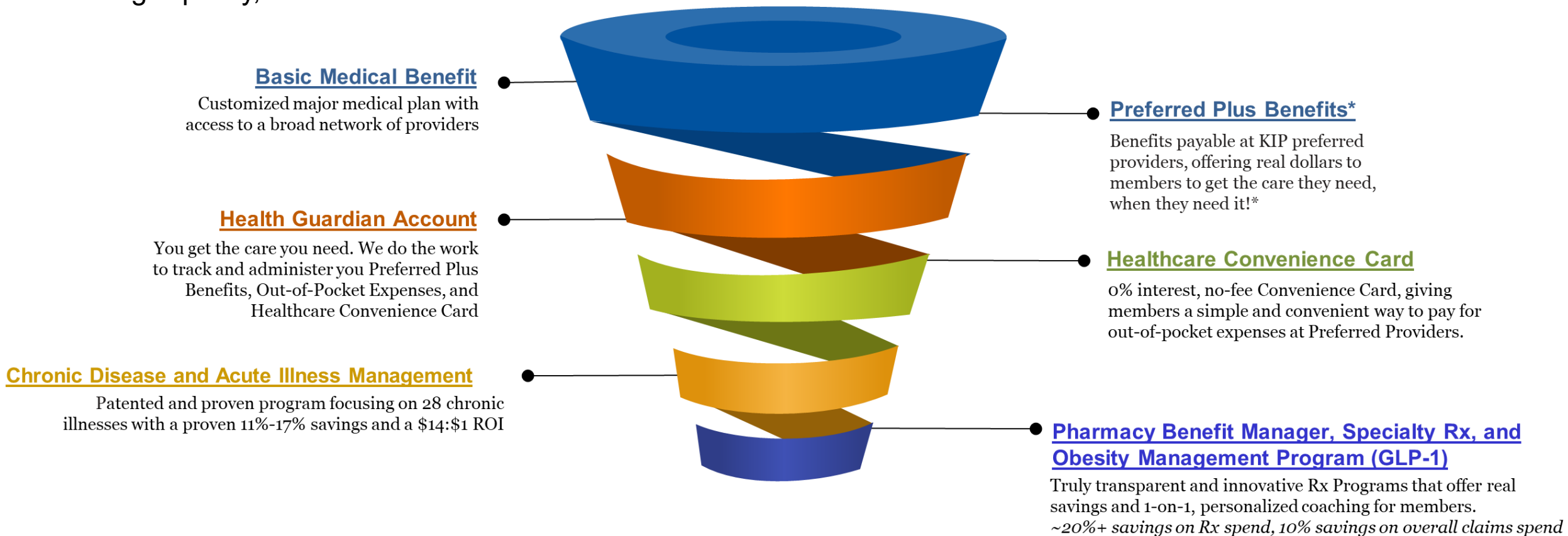
KEY BENEFIT
ADMINISTRATORS

Explanation of Benefits	
Heart Attack, Cardiac Surgery	
THE CHARGES	
Medical Bills	\$25,000
Healthcare out-of-pocket due to provider	\$4,500
Since your out of pocket maximum has been met, the rest of your eligible charges this year will be paid at 100%	
THE CREDITS	
OP Radiology/X-Ray	\$200
OP Diagnostic Labs	\$100
Preferred Hospital Stay (\$500/day, 5 days)	\$2,500
Critical Illness Benefit	\$2,000
Health Screening Benefit	\$125
Benefit Total	\$4,925
Out of Pocket Paid to Hospital	(\$4,500)
Balance Paid to HGA	\$425

The Key Incentive Plan

Affordability Without Compromise

The **Key Incentive Plan** is a one-of-a-kind solution, empowering employers to take back control of their healthcare programs, that offers freedom of choice when choosing providers in your local community, while incenting members to choose high-quality, cost-effective care.



Weight Management (GLP-1s) that Don't Break the Bank

The GLP-1 medication market has experienced explosive growth, fundamentally reshaping healthcare spending across America. Prime Therapeutics reports that per-member-per-month spend has tripled by March 2025, with these breakthrough medications now representing approximately 15% of total pharmacy spend. More than 1 in 50 members filed a GLP-1 claim in Q1 2025 alone, demonstrating unprecedented demand for these life-changing treatments.

The Challenge

Traditional GLP-1 medications cost between \$1,089-\$1,349 monthly, creating monthly, creating unsustainable budget pressures for employers while limiting employee access to essential treatments.

Our Solution

Key Benefits secures generic GLP-1 medications for just \$249 per month, enabling comprehensive workforce health while maintaining strict budget control and financial stability.

The Impact

Deliver a healthier population, increased employee satisfaction, and sustainable costs that support long-term organizational success and success and employee wellbeing.

Proven Results: \$7M Annual Savings

One major city spending \$11 million annually on GLP-1 medications achieved \$7 million in cost savings through our comprehensive approach. This dramatic reduction demonstrates how strategic sourcing combined with clinical support creates sustainable, scalable solutions for large employers facing escalating pharmaceutical costs.

Beyond Medication: Comprehensive Care

Our program extends far beyond cost-effective medication access. Members receive specialized coaching twice monthly, focusing on maintaining muscular strength, engaging in moderate exercise, and developing sustainable lifestyle changes that support long-term weight maintenance after GLP-1 treatment completion.

270

Health Conditions

Created or exacerbated by obesity according to Cleveland Clinic research

\$8.5K

Additional Spend

Average annual healthcare costs above normal for obese individuals (KFF)

✔ **Transform Your Healthcare Strategy:** Join forward-thinking employers who are providing comprehensive weight management solutions while protecting their bottom line. Our approach combines clinical excellence with fiscal responsibility, creating a win-win for organizations and their valued employees.

WHY MU HEALTH CARE?



DIRECT-TO-EMPLOYER STRATEGY:
ONLY AVAILABLE TO *PREMIER* HEALTH SYSTEMS



STRONG Brand Quality

EXTENSIVE Patient Access

AFFORDABLE Value Pricing

EXCELLENT Care Quality

MU HEALTHCARE IS PATIENT RECOMMENDED

Health System	HCAHPS Recommendation Percentile	CMS Stars Percentile	Mortality Percentile	Readmission Percentile	KBA Brand Quality Score
Boone Hospital Center	83	100	81	91	74
University of Missouri Health Care	64	35	66	73	66
SSM Health	54	60	69	61	65

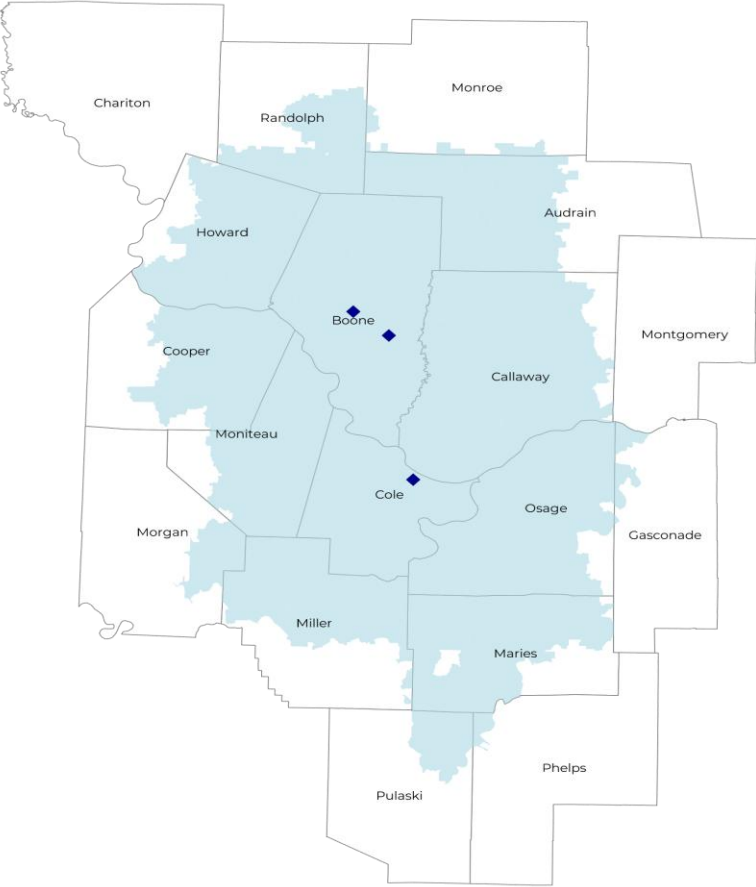
Based on quality measures and HCAHPS feedback, KBA estimates that MU Healthcare has the strongest brand quality in Audrain, Boone, Callaway, Cole, Cooper, Gasconade, Howard, Maries, Miller, Moniteau, Morgan, Osage, Pulaski, and Randolph Counties.

SIGNIFICANT OPPORTUNITY FOR PLAN GROWTH

Researched for the below highlighted service area

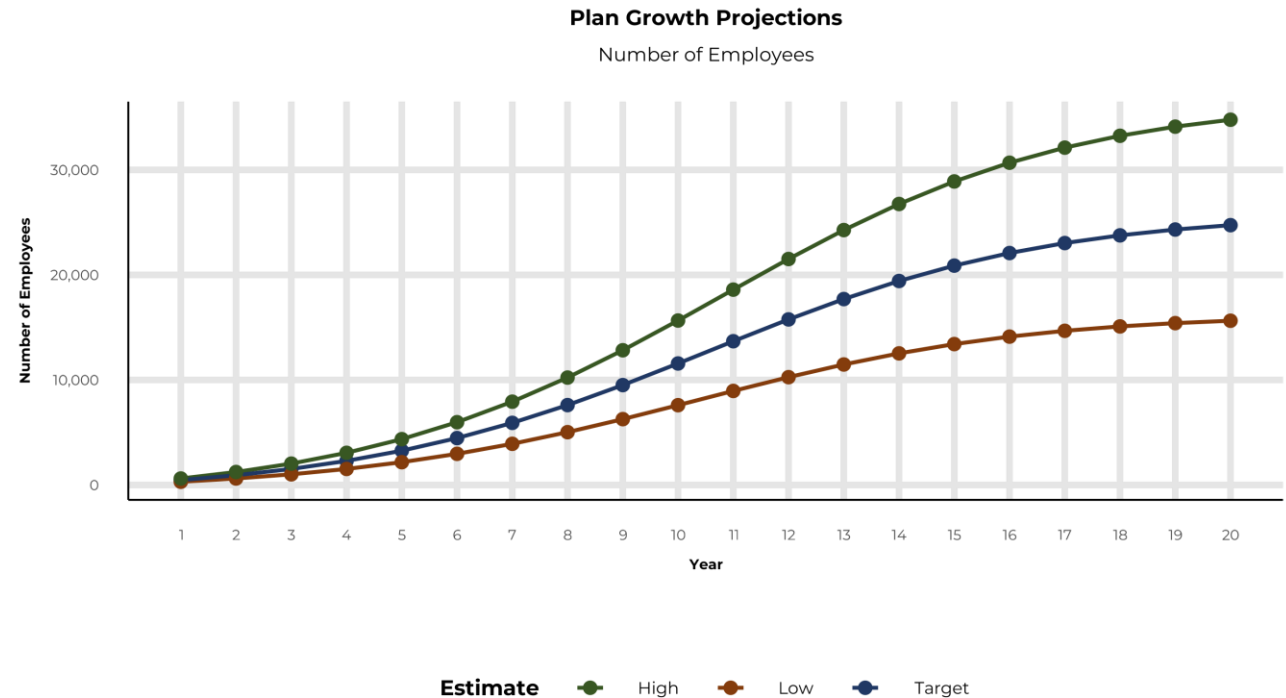
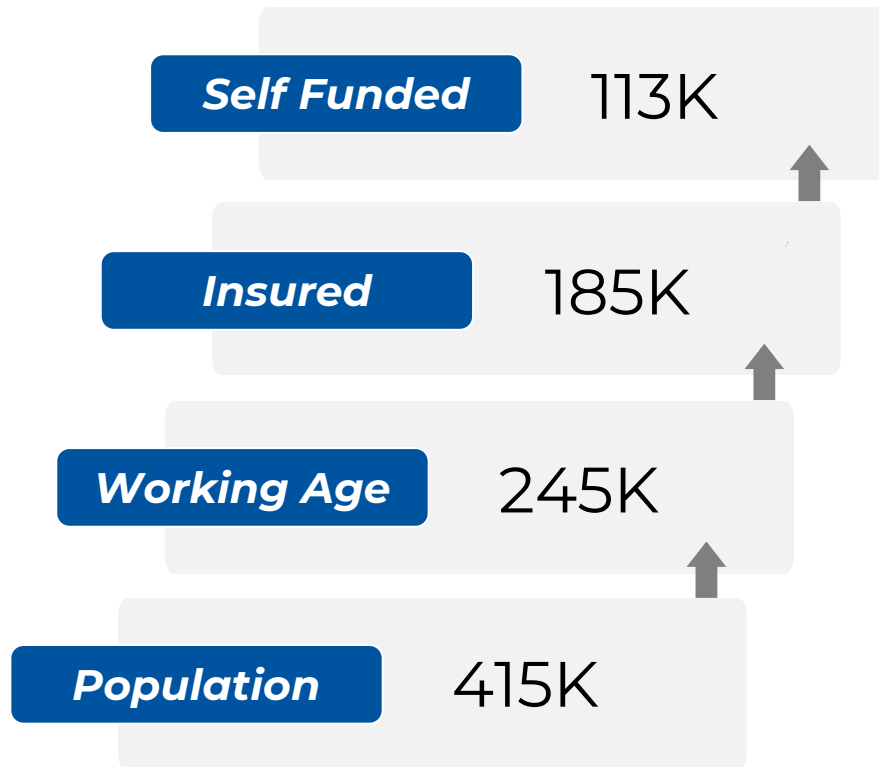
Target Market

400+ Employers ➤ 95,000+ Employees ➤ 170,000+ Members



	Employer Size Group (# of Employees)					
Commercial Employers With Headquarters In Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	259	108	53	38	9	5
Total Employees at Employers Headquartered in Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	19,400	16,200	15,900	26,600	18,000	30,000
Total Members with Employers Headquartered in Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	34,900	29,200	28,600	47,900	32,400	54,000

PLAN POTENTIAL GROWTH PATHS



Estimates includes the MU Healthcare service area in Audrain, Boone, Callaway, Cole, Cooper, Gasconade, Howard, Maries, Miller, Moniteau, Morgan, Osage, Pulaski, and Randolph Counties

GROWING POWERFUL, LASTING PROFITABILITY

Early Stage

Gaining Name Recognition, Early Growth

New Revenue:
\$6.33M

New Profit:
\$1.31M

Growth Stage

Strong Reputation, Strong Growth

New Revenue:
\$38.36M

New Profit:
\$9M

Mature Stage

High Profitability, Market Saturation

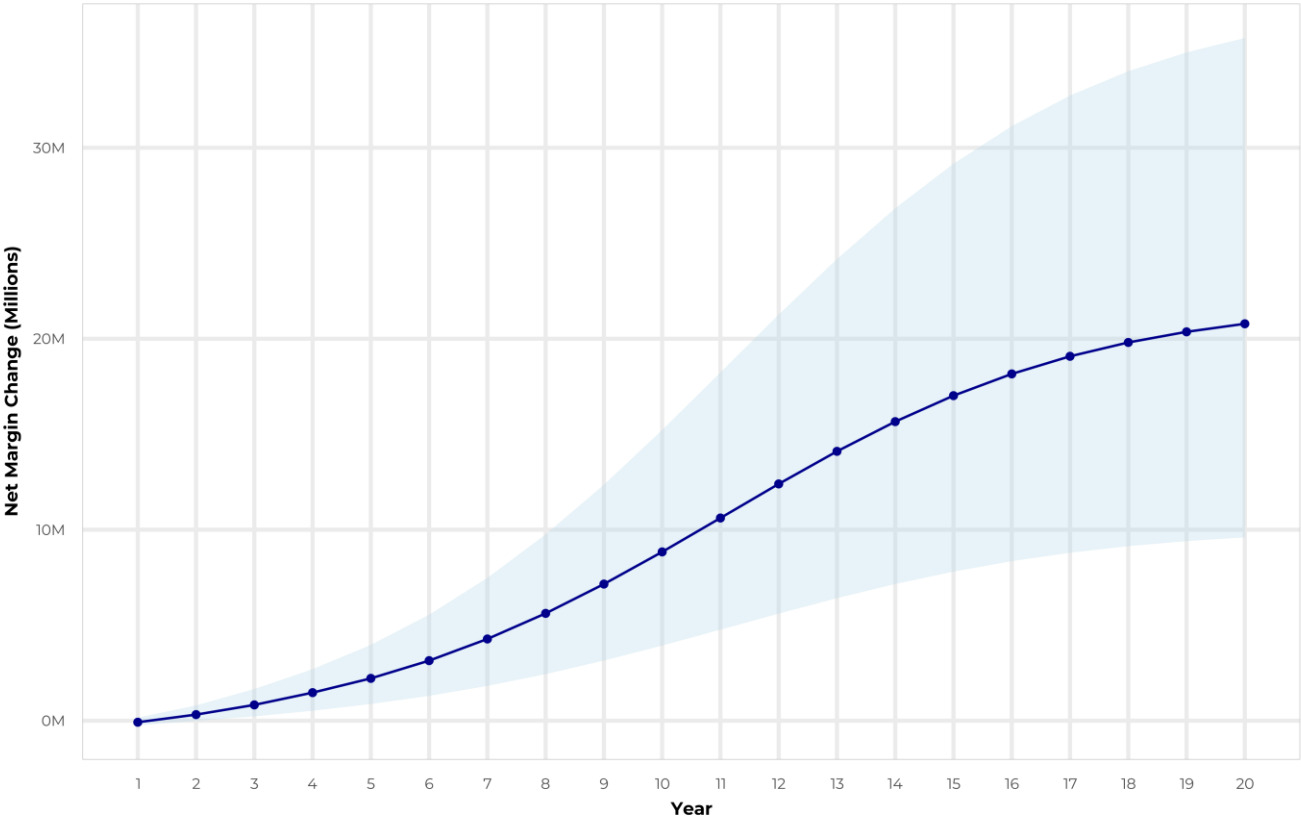
New Revenue:
\$79.02M

New Profit:
\$18.7M

Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

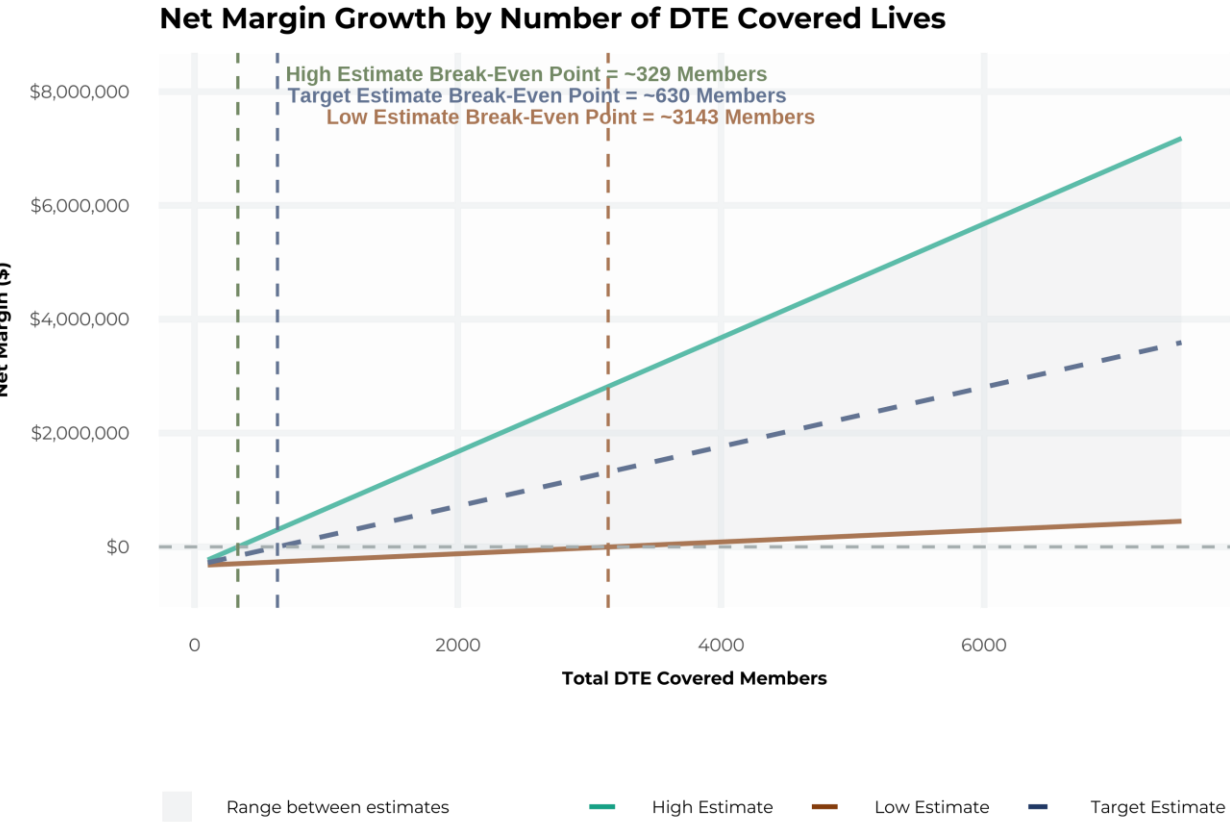
60%+ Within-Plan Steerage and a Commercial Margin of 21.6%





FINANCIAL PROJECTIONS & ASSUPTIONS

MODEL ASSUMPTIONS AND BREAK EVEN



Patient Steerage Assumptions

KBA currently estimates that MU Healthcare has 64% inpatient market share, and 48% outpatient market compared to hospitals/clinics in the MU Healthcare service area (see slide 8). The DTE tiered structure will create immediate steerage, with additional steerage over time from patient bonus benefits and the patient card (see slide 34 for estimates of target steerage).

Low	High
10% below target	10% above target

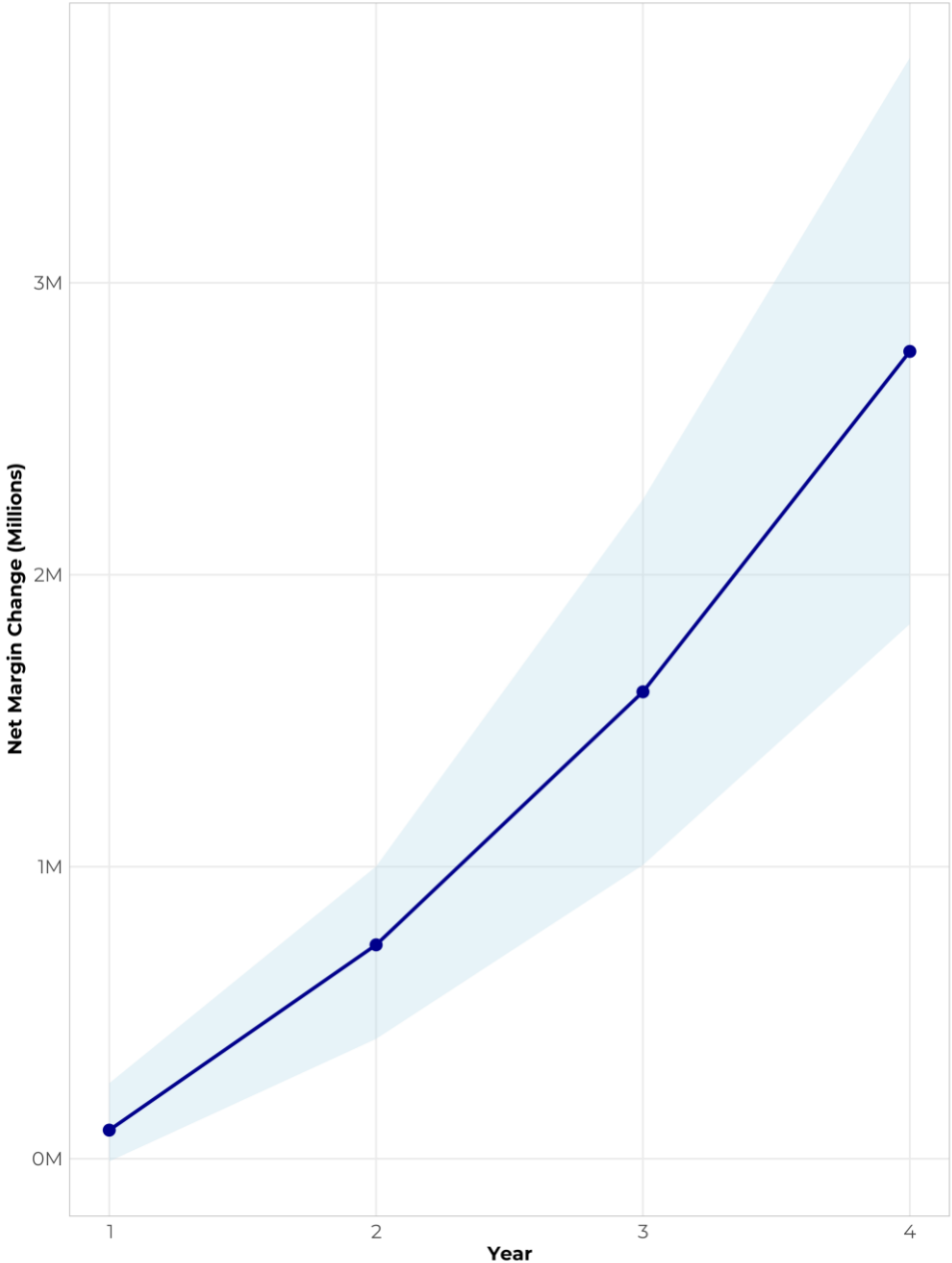
Commercial Margin Assumptions

KBA observes a MU Healthcare 29% commercial margin. Conservatively, we model the DTE to be even with commercial rates given prices 10% below market, but with a 10 percentage-point increase through friendlier billing practices and improved patient collections with the patient convenience card.

Low	Target	High
20.7%	23.2%	25.7%

FOUR-YEAR PROJECTIONS

Net Margin Change Over Years
Variation by Member Growth With Target Estimate Assuming
60%+ Within-Plan Steerage and a Commercial Margin of 21.6%



Target Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	800	1,700	2,700	4,100
Incremental Margin	418.2K	892.1K	1,746.7K	2,895.8K
Network Access Fee	9.6K	20.4K	32.4K	49.2K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	97.8K	732.5K	1,599.1K	2,765.0K

High Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	1,100	2,200	3,700	5,500
Incremental Margin	575.0K	1,154.4K	2,393.6K	3,884.6K
Network Access Fee	13.2K	26.4K	44.4K	66.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	258.2K	1,000.8K	2,258.0K	3,770.6K

Low Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	600	1,100	1,800	2,800
Incremental Margin	313.7K	577.2K	1,164.5K	1,977.6K
Network Access Fee	7.2K	13.2K	21.6K	33.6K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	-9.1K	410.4K	1,006.1K	1,831.2K

DTE Covered Members Financial Analysis

Target Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 21.6%.

DTE Members	Pre-DTE Revenue	Pre-DTE Margin	DTE Revenue	DTE Incr. Revenue	DTE Total Margin	DTE Incr. Margin	Net Margin Less DTE Fees
100	\$ 568K	\$ 132K	\$ 788K	\$ 220K	\$ 183K	\$ 51K	\$ -278K
500	\$ 2,840K	\$ 659K	\$ 3,941K	\$ 1,101K	\$ 914K	\$ 255K	\$ -69K
1,000	\$ 5,681K	\$ 1,318K	\$ 7,882K	\$ 2,202K	\$ 1,829K	\$ 511K	\$ 193K
2,500	\$ 14,202K	\$ 3,295K	\$ 19,706K	\$ 5,504K	\$ 4,572K	\$ 1,277K	\$ 977K
10,000	\$ 56,807K	\$ 13,179K	\$ 78,823K	\$ 22,016K	\$ 18,287K	\$ 5,108K	\$ 4,898K
20,000	\$ 113,613K	\$ 26,358K	\$ 157,646K	\$ 44,032K	\$ 36,574K	\$ 10,215K	\$ 10,125K

- NOTE: Please see detailed financial analysis and assumptions used starting on page 27.
- Financial projections do not include 340B contribution margin, which could represent \$136 PMPM by optimizing 340B through Rx plan design, MU Healthcare's approved site(s) with network integration, and development of MU Healthcare's proprietary PBM program.

ESTIMATION OF MU HEALTHCARE'S 2022 PAYER MIX

Payer Group	Total Inpatient Days	Revenue	Payer Mix
All MU Healthcare	161,469	\$1,591,541K	100%
Medicare	38,689	\$208,090K	23%
Medicaid	23,741	\$266,425K	15%
Other Government Pay	0		
No-Pay, Charity Care	4,958	-\$55,520K	3%
Commercial/Self Pay	94,081	\$1,119,214K	59%

Values in blue are taken from the 2021 Medicare Provider Cost Report (<https://data.cms.gov/provider-compliance/cost-report/hospital-provider-cost-report/data>)

Values in yellow are taken from Definitive Healthcare data.

Total Medicare patient revenue is reported by CMS in the Medicare Provider Analysis and Review Report (MedPAR)

Estimating Payer Mix

- Medicare, Medicaid, and other government payer mixes are estimated as the percentage of inpatient days for each group divided by total inpatient days.
- No-pay and charity care is estimated as the percentage of no-pay/charity care compared to total overall revenue.
- Commercial/self pay is assumed to be the rest of the total payer mix share.

Estimating Total Inpatient Days

- Total inpatient days for 'No Pay' and 'Commercial' are estimated to be the payer mix estimates for "No Pay, Charity Care" and "Commercial/Self Pay" multiplied by the total number of inpatient days.

In other analysis, the outpatient payer mix is assumed to mirror the inpatient payer mix.

ESTIMATION OF MARGIN FOR COMMERCIAL VS. GOVERNMENT PAY

The RAND data for MU Healthcare estimates commercial insurance reimbursements at 246% of Medicare Rates

Estimating Commercial Margin

(Calculated through counterfactual of assuming all CMS/Government pay was paying at commercial rates)

Commercial Revenue	\$1,119,214K
Gov-pay revenue at 246% current rate	\$1,167,684K
Total Revenue	\$2,286,898K
Total Costs	\$1,628,984K
Net Income	\$657,914K
	29%

Estimating Gov-Pay Margin

(Calculated through counterfactual of assuming all commercial pay was paying at Medicare rate)

Commercial Revenue	\$454,818K
Divided by 246%	
Gov-pay revenue	\$474,515K
Total Revenue	\$929,333K
Total Costs	\$1,628,984K
Net Income	\$ -699,651K
	-75%

ESTIMATING MARKET SHARE

Hospital Name	Number of Beds	Inpatient Total Charges	Outpatient Total Charges	Inpatient Percent of Market Total	Outpatient Percent of Market Total (Assuming 66% of Physicians Employed by Hospitals)
University Hospital	726	\$2,398,160.9K	\$3,339,080.7K	63%	48%
Rusk Rehabilitation Hospital	60	\$ 37,836.0K	\$ NAK	1%	NA%
SSM Health St Marys Hospital - Jefferson City	154	\$ 201,847.5K	\$ 325,119.0K	5%	5%
Boone Hospital Center	270	\$ 830,430.0K	\$ 701,958.8K	22%	10%
Moberly Regional Medical Center	39	\$ 104,488.7K	\$ 225,575.3K	3%	3%
Harry S Truman Memorial Veterans Hospital	126	\$ NAK	\$ NAK	NA%	NA%
Landmark Hospital of Columbia	39	\$ 41,311.9K	\$ NAK	1%	NA%
Fulton State Hospital	49	\$ 127,961.6K	\$ 5.7K	3%	<1%
CenterPointe Hospital of Columbia	80	\$ 63,025.2K	\$ 1,946.2K	2%	<1%

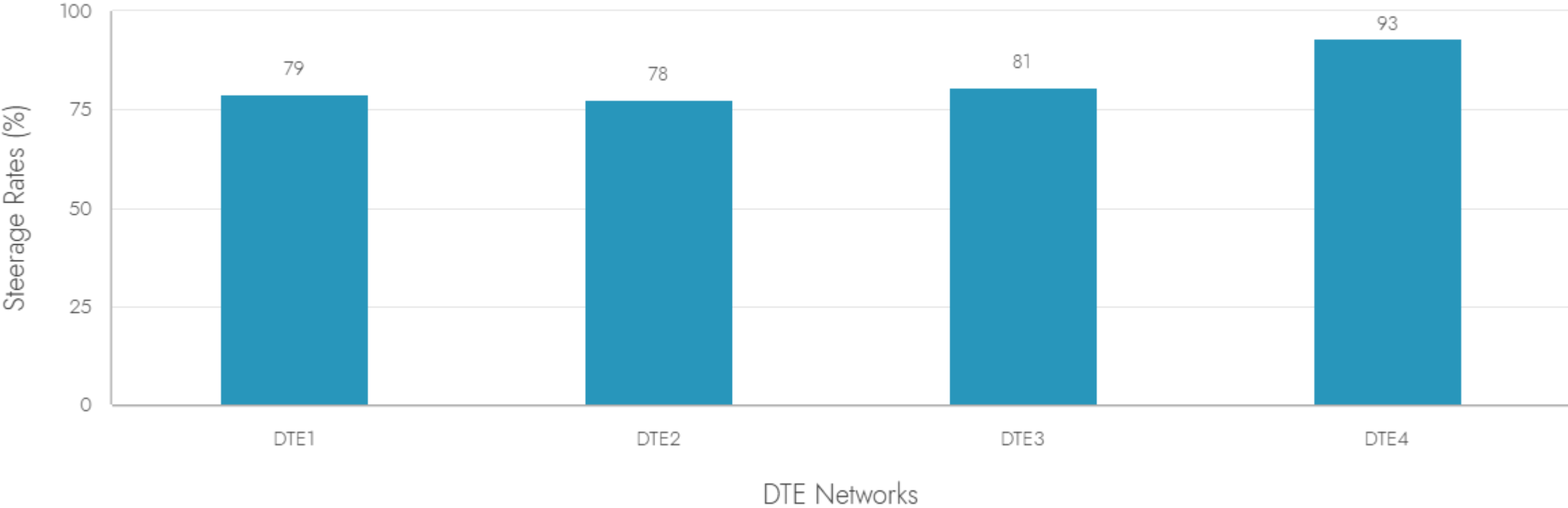
64%
Inpatient
Market
Share

48%
Outpatient
Market
Share

For outpatient market share, it is estimated that only 66% of local physicians are employed by area hospitals (Research from the Physician Advocacy Institute:
<https://www.physiciansadvocacyinstitute.org/Portals/0/assets/docs/PAI-Research/PAI-Avalere%20Physician%20Employment%20Trends%20Study%202019-2023%20Final.pdf?ver=uGHF46uIGSeZgYXMKFyYvw%3d%3d>)

Hospital Metrics			
Metric	Capital Region Medical Center	University Hospital	Rusk Rehabilitation Hospital
Total Inpatient Patient Revenue	\$43,844K	\$598,800K	\$25,736K
Total Outpatient Patient Revenue	\$180,162K	\$754,656K	\$0K
Total Medicare Revenue	\$32,311K	\$173,465K	\$10,313K
Estimated Medicaid and Other Government-Pay Revenue	\$11,850K	\$242,189K	\$0K
Estimated Commercial Revenue	\$179,832K	\$939,538K	\$15,423K
Total Uncompensated Care	\$17,870K	\$66,122K	\$0K
Estimated Inpatient Costs	\$51,620K	\$611,550K	\$24,756K
Estimated Outpatient Costs	\$212,116K	\$770,725K	\$0K
Total Operating Expenses	\$263,736K	\$1,382,276K	\$24,756K
Net Income	\$-39,742K	\$-27,083K	\$980K
Estimated Payer Mix Percent Medicare	31.7%	22.9%	33.6%
Estimated Payer Mix Percent Medicaid	8.1%	14.5%	16.5%
Estimated Payer Mix Percent Other Government-Pay	0.0%	0.0%	0.0%
Payer Mix Percent No Pay	7.4%	4.7%	0.0%
Estimated Payer Mix Percent Commercial	52.7%	58.0%	49.9%
Estimated Commercial Profit Margin	8.6%	29.6%	39.3%
1. Most data is taken from the CMS provider cost report			
2. Medicare costs are taken from the Medicare Provider Analysis and Review (MedPAR)			
3. Data with 'Estimated' in the title are derived from other data points.			
4. Market Share estimates based on total patient revenues with area hospitals. Ambulatory market share based on regional estimates of the percentage of local physicians employed by area hospitals. See Physician Advocacy Institute			

STEERAGE RATES FOR CURRENT DTE NETWORKS



The average Tier 1 Steerage is 83%.

PROJECTED PLAN GROWTH

Year	Employees	Members	Percent of Members Choosing MU Healthcare for Inpatient Care	Percent of Members Choosing MU Healthcare for Outpatient Care
1	500	900	80%	70%
2	900	1,600	80%	70%
3	1,500	2,700	85%	75%
4	2,300	4,100	85%	78%
5	3,300	5,900	86%	79%
6	4,500	8,100	86%	79%
7	5,900	10,600	86%	80%
8	7,600	13,700	86%	80%
9	9,500	17,100	86%	80%
10	11,600	20,900	87%	81%
11	13,700	24,700	87%	81%
12	15,800	28,400	87%	82%
13	17,700	31,900	87%	82%
14	19,400	34,900	87%	82%
15	20,900	37,600	87%	83%
16	22,100	39,800	88%	83%
17	23,000	41,400	88%	83%
18	23,800	42,800	88%	84%
19	24,300	43,700	88%	84%
20	24,700	44,500	88%	84%

KBA currently estimates that MU Healthcare has 66% inpatient market share, and 50% outpatient market compared to hospitals/clinics in the MU Healthcare service area (see slide 8).

Based on previous DTE experience, KBA expects a significant bump in patient steerage to MU Healthcare.

Additional steerage will occur from additional steerage patient benefits and the patient card.

High estimates start nearly 10% higher than target with low estimates at 10% lower than target

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 4,100 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	\$ 7,767,266	\$ 10,581,855	\$ 4,941,636	\$ 23,290,757
Net Income‡	\$ 1,802,006	\$ 2,454,990	\$ 1,146,460	\$ 5,403,456

With DTE	Inpatient	Outpatient	Professional	Total
Revenue°	\$ 10,351,020	\$ 17,184,506	\$ 8,025,018	\$ 35,560,544
Net Income*	\$ 2,401,437	\$ 3,986,805	\$ 1,861,804	\$ 8,250,046

† Assuming proportional revenues from market shares of 64% (inpatient) and 48% (outpatient)

‡ Conservatively assuming a commercial margin of 23.2% (80% of the estimated commercial margin)

° Assuming 85% inpatient and 78% market steerage based on KBA narrow network DTE experience

* Conservatively assuming a commercial margin of 23.2%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member credit card.

Total Increase in Net Income	\$ 2,846,591
Network Access Fee (Revenue) of \$1 PMPM	\$ 49,200
Total Profit Growth	\$ 2,851,391
Minus Implementation Fee (One-Time)	-\$ 0
Minus Sales and Marketing (Annual – Ongoing)	-\$ 180,000
Total	\$ 2,715,791

Target Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 21.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Inpatient Revenue	\$947.2	\$1,894.5	\$9,472.3	\$47,361.4
Pre-DTE Group Inpatient Margin	\$219.8	\$439.5	\$2,197.6	\$10,987.8
DTE Group Inpatient Revenue	\$1,181.4	\$2,362.9	\$11,814.3	\$59,071.7
DTE Group Inpatient Margin	\$274.1	\$548.2	\$2,740.9	\$13,704.6
DTE Group Incremental Inpatient Revenue	\$234.2	\$468.4	\$2,342.1	\$11,710.3
DTE Group Incremental Inpatient Margin	\$54.3	\$108.7	\$543.4	\$2,716.8
Pre-DTE Group Outpatient Revenue	\$1,290.5	\$2,580.9	\$12,904.7	\$64,523.5
Pre-DTE Group Outpatient Margin	\$299.4	\$598.8	\$2,993.9	\$14,969.5
DTE Group Outpatient Revenue	\$1,881.2	\$3,762.4	\$18,812.0	\$94,060.0
DTE Group Outpatient Margin	\$436.4	\$872.9	\$4,364.4	\$21,821.9
DTE Group Incremental Outpatient Revenue	\$590.7	\$1,181.5	\$5,907.3	\$29,536.5
DTE Group Incremental Outpatient Margin	\$137.0	\$274.1	\$1,370.5	\$6,852.5
Pre-DTE Group Professional Revenue	\$602.6	\$1,205.3	\$6,026.4	\$30,131.9
Pre-DTE Group Professional Margin	\$139.8	\$279.6	\$1,398.1	\$6,990.6
DTE Group Professional Revenue	\$878.5	\$1,757.0	\$8,785.0	\$43,925.2
DTE Group Professional Margin	\$203.8	\$407.6	\$2,038.1	\$10,190.7
DTE Group Incremental Professional Revenue	\$275.9	\$551.7	\$2,758.7	\$13,793.3
DTE Group Incremental Professional Margin	\$64.0	\$128.0	\$640.0	\$3,200.0
Net Margin Less DTE Fees	\$-68.6	\$192.8	\$2,283.9	\$12,739.3

Target Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 21.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$2,840.3	\$5,680.7	\$28,403.4	\$142,016.8
Pre-DTE Group Total Margin	\$659.0	\$1,317.9	\$6,589.6	\$32,947.9
DTE Group Total Revenue	\$3,941.1	\$7,882.3	\$39,411.4	\$197,057.0
DTE Group Incremental Total Revenue	\$1,100.8	\$2,201.6	\$11,008.0	\$55,040.2
Incremental Total Revenue by Member	\$2.2	\$2.2	\$2.2	\$2.2
DTE Group Total Margin	\$914.3	\$1,828.7	\$9,143.4	\$45,717.2
DTE Group Incremental Total Margin	\$255.4	\$510.8	\$2,553.9	\$12,769.3
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-68.6	\$192.8	\$2,283.9	\$12,739.3

High Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 26.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$2,840.3	\$5,680.7	\$28,403.4	\$142,016.8
Pre-DTE Group Total Margin	\$659.0	\$1,317.9	\$6,589.6	\$32,947.9
DTE Group Total Revenue	\$4,488.6	\$8,977.3	\$44,886.5	\$224,432.4
DTE Group Incremental Total Revenue	\$1,648.3	\$3,296.6	\$16,483.1	\$82,415.6
DTE Group Total Margin	\$1,153.6	\$2,307.2	\$11,535.8	\$57,679.1
DTE Group Incremental Total Margin	\$494.6	\$989.2	\$4,946.2	\$24,731.2
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$170.6	\$671.2	\$4,676.2	\$24,701.2

Low Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 16.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$2,840.3	\$5,680.7	\$28,403.4	\$142,016.8
Pre-DTE Group Total Margin	\$659.0	\$1,317.9	\$6,589.6	\$32,947.9
DTE Group Total Revenue	\$3,405.6	\$6,811.2	\$34,055.9	\$170,279.6
DTE Group Incremental Total Revenue	\$565.3	\$1,130.5	\$5,652.5	\$28,262.7
DTE Group Total Margin	\$705.0	\$1,409.9	\$7,049.6	\$35,247.9
DTE Group Incremental Total Margin	\$46.0	\$92.0	\$460.0	\$2,300.0
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-278.0	\$-226.0	\$190.0	\$2,270.0



IMPLEMENTATION AND OPERATIONS

Claims Processing Excellence

- Delivering a positive member and provider experience starts with accurate, efficient, and transparent claims processing
- **Key Benefit Administrators leverages:**
 - Proven technology
 - Streamlined workflows
 - A team dedicated to first pass accuracy
- **We Consistently:**
 - Exceed industry standard turnaround times and quality metrics
 - Reduce administrative burdens
 - Maintain high member and provider satisfaction
- **Strategic Partnerships:**
 - We partner with **Smart Data Solutions** for clearinghouse services to empower providers with electronic claims submission and transparency tools.
 - Currently 82% of our claim volume is received electronically.
 - Ongoing efforts to increase electronic submissions:
 - Analyze paper and non-electronic claims
 - Proactively engage providers for electronic claim setup
 - We also partner with top tier vendors to provide cost containment solutions to include:
 - Out of network discounts
 - High dollar bill reviews
 - Specialized networks for high-cost conditions such as transplants and dialysis.



Claims Statistics

Claims	Industry Standard	Corporate Goal	Self-Funded Medical Achieved*
Turn Around Time	10 calendar days	7 calendar days	4.25 calendar days
% Paid in 10 days	80%	85%	86.7%
Financial Quality	99%	99%	99.6%
Payment Accuracy	NA	95%	97.4%

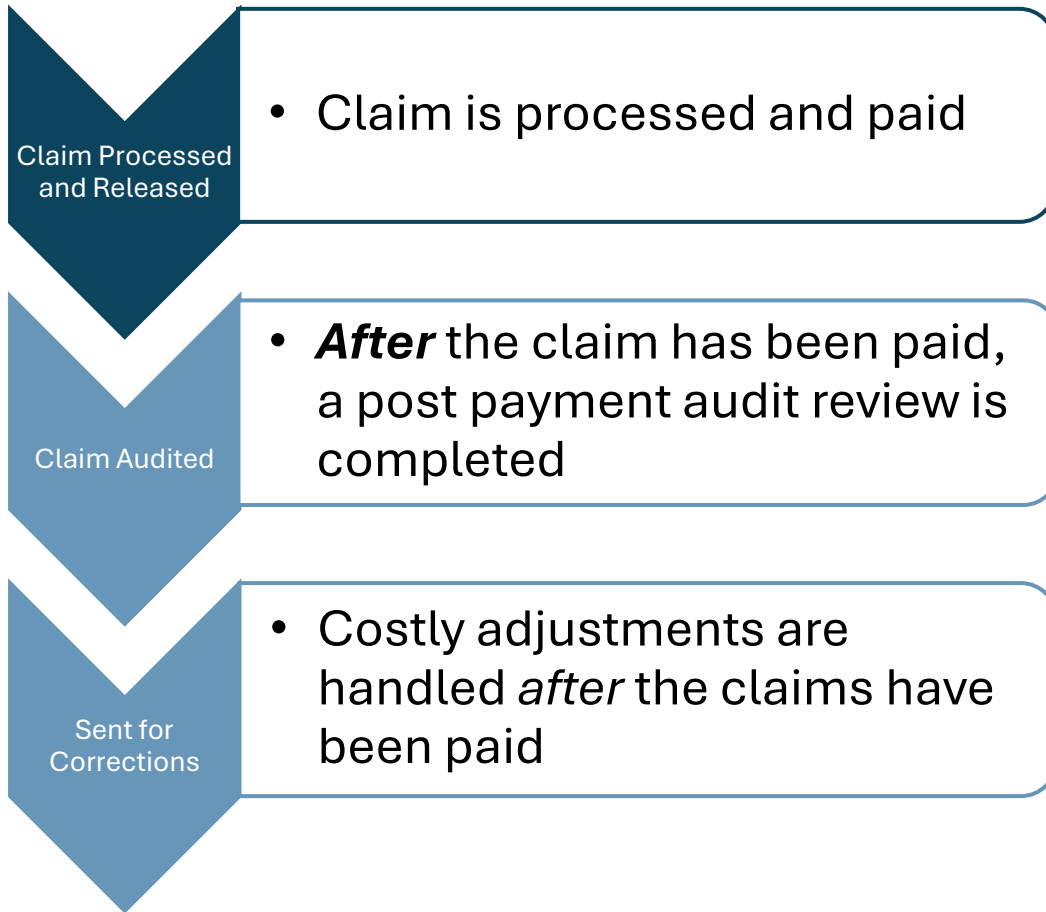
* Data achieved is for Jan-Jun 2025 calendar year

- Groups are provided with a dedicated specialist to handle escalated issues and concerns promptly.
- We offer comprehensive, interactive training for claims and customer service staff to prepare them for their roles, using a collaborative LMS system to ensure consistency and accuracy in KBA processes.



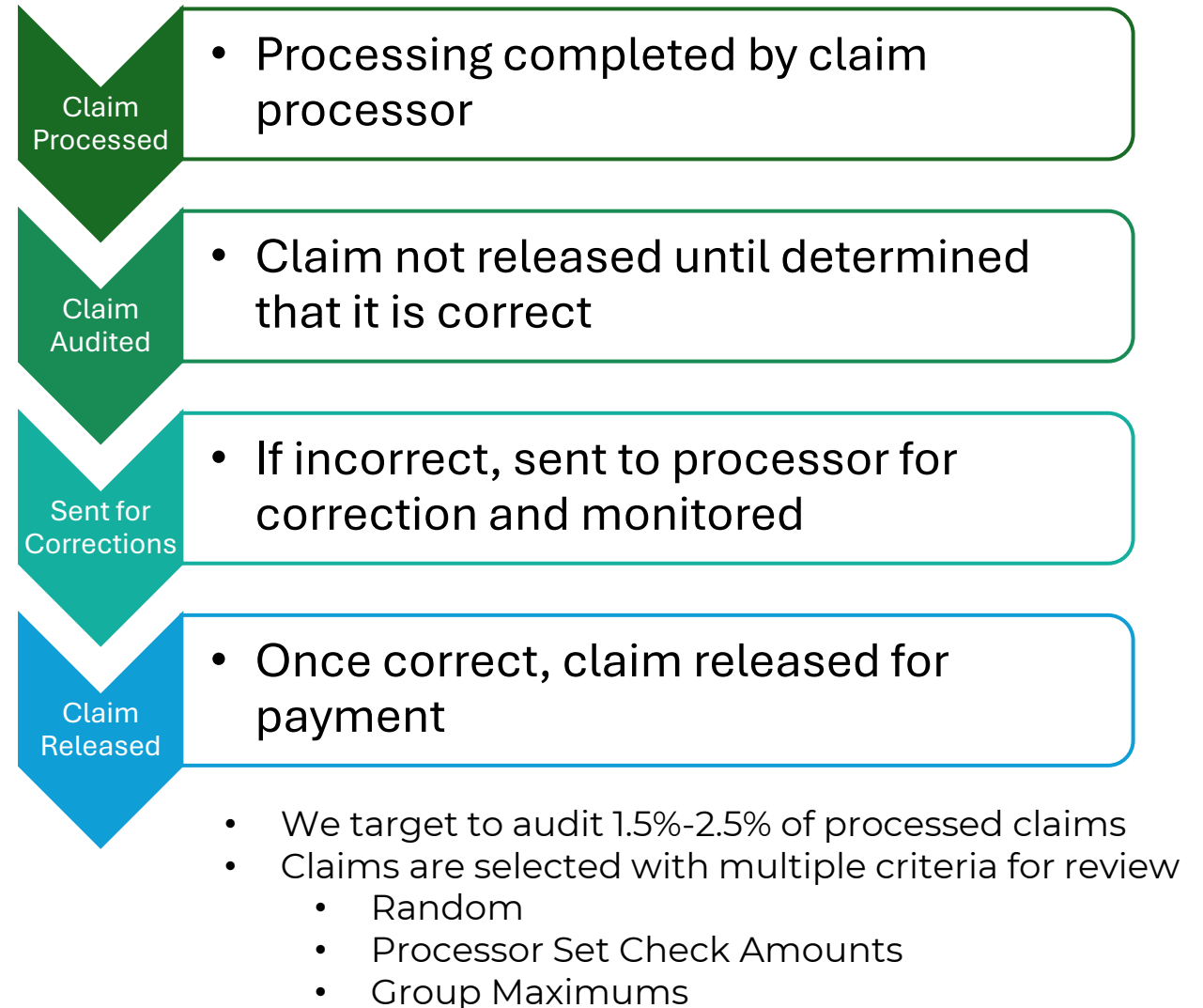
Industry Claim Audits

Post Payment Reviews



KBA Claim Audits

Pre Payment Reviews



KBA 2023-2024 Total Quality Management Statistics

300+
TOTAL QUALITY MEASURES

50,000+
UNIQUE AUDITS ANNUALLY

\$200M+
PREMIUM & CLAIMS REVIEWED ANNUALLY

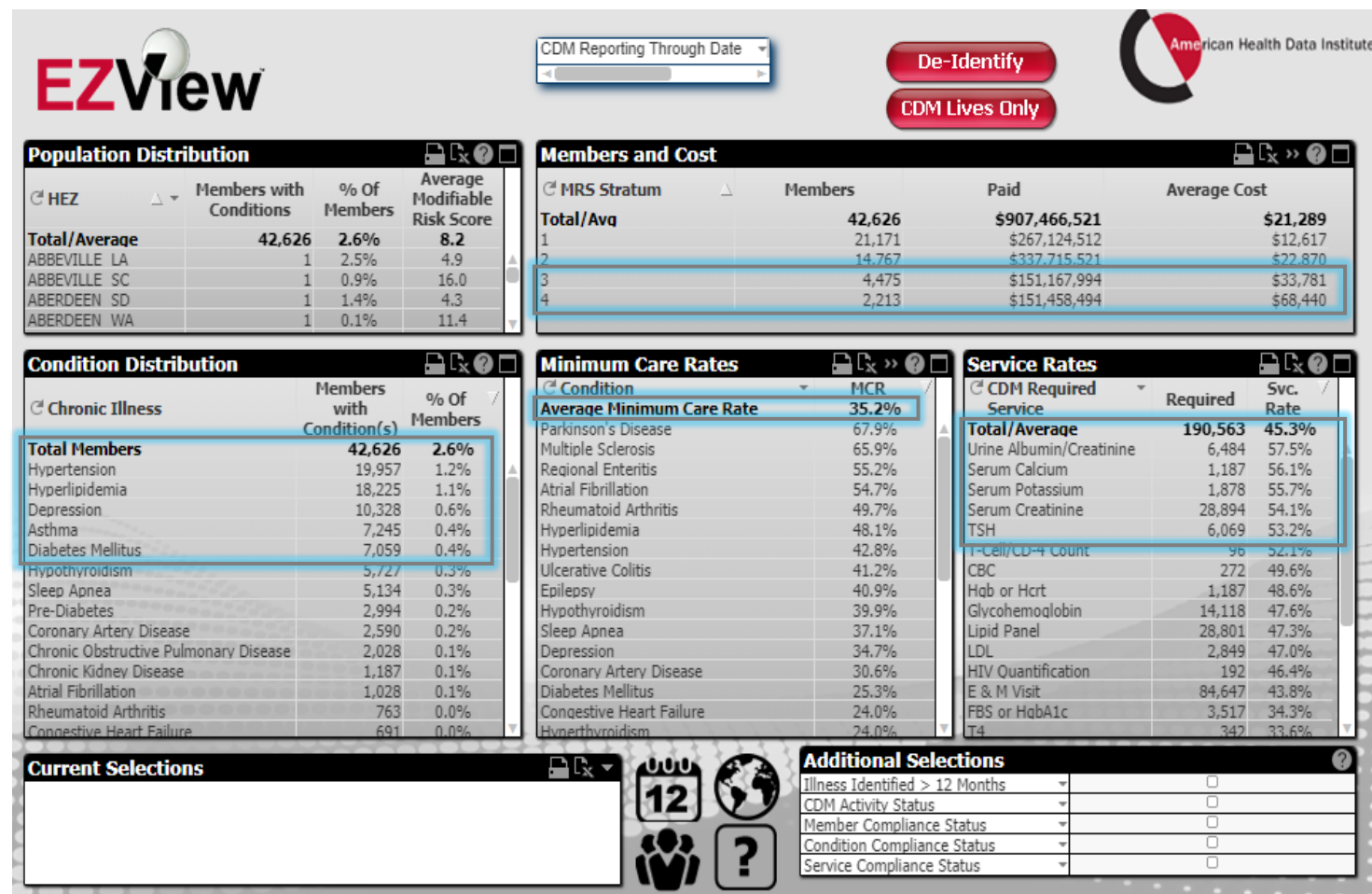
New Business / Renewals					Enrollment Center				
New Case Accuracy	2023 KBA 99.86%	2024 KBA 100.00% ↑	INDUSTRY N/A	GOAL 99.75%	Soft Skills Audit	2023 KBA 99.43%	2024 KBA 99.22% ↓	INDUSTRY N/A	GOAL 98.00%
Renewal Case Accuracy	2023 KBA 99.96%	2024 KBA 100.00% ↑	INDUSTRY N/A	GOAL 99.75%	Procedural Skills Audit	2023 KBA 98.26%	2024 KBA 98.23% ↓	INDUSTRY N/A	GOAL 97.00%
Fulfillment Materials Audit	2023 KBA 99.99%	2024 KBA 99.98% ↓	INDUSTRY N/A	GOAL 100%					

Billing & Eligibility Quality					Claims Quality				
Eligibility Accuracy	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Turn Around Time (Days)	2023 KBA 3.79	2024 KBA 6 ↓	INDUSTRY 10	GOAL 7
Plans/Rates/Coverage Audits	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	% Paid within 10 Bus Days	2023 KBA 88.90%	2024 KBA 87.34% ↓	INDUSTRY 80.00%	GOAL 85.00%
Cash Receipts Logging	2023 KBA 99.97%	2024 KBA 99.78% ↓	INDUSTRY N/A	GOAL 98.00%	Financial Accuracy	2023 KBA 99.30%	2024 KBA 99.44% ↑	INDUSTRY 99.00%	GOAL 99.00%
Cash Posting Accuracy	2023 KBA 99.96%	2024 KBA 99.78% ↓	INDUSTRY N/A	GOAL 97.00%	Procedural Accuracy	2023 KBA 99.00%	2024 KBA 99.16% ↑	INDUSTRY N/A	GOAL 95.00%
					% of Claims Audited	2023 KBA 3.77%	2024 KBA 3.80% ↑	INDUSTRY 1%	GOAL 3%

Customer Experience Quality									
Average Speed to Answer	2023 KBA 22 sec	2024 KBA 44 sec ↓	INDUSTRY <45s	GOAL <30s	% Answered w/in 45 Sec	2023 KBA 84.90%	2024 KBA 75.00% ↓	INDUSTRY 80-60%	GOAL 80-30%
Soft Skills Audit	2023 KBA 99.50%	2024 KBA 99.54% ↑	INDUSTRY N/A	GOAL 98.00%	Procedural Audit	2023 KBA 99.22%	2024 KBA 99.10% ↓	INDUSTRY N/A	GOAL 97.00%
Email Correspondence Audit	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 98.00%	Web Chat Corresp. Audit	2023 KBA 99.64%	2024 KBA 99.50% ↓	INDUSTRY N/A	GOAL 98.00%
Abandonment Rate	2023 KBA 1.00%	2024 KBA 1.78% ↓	INDUSTRY <5%	GOAL <1%					

TRANSPARENT ANALYTICS ACCESS

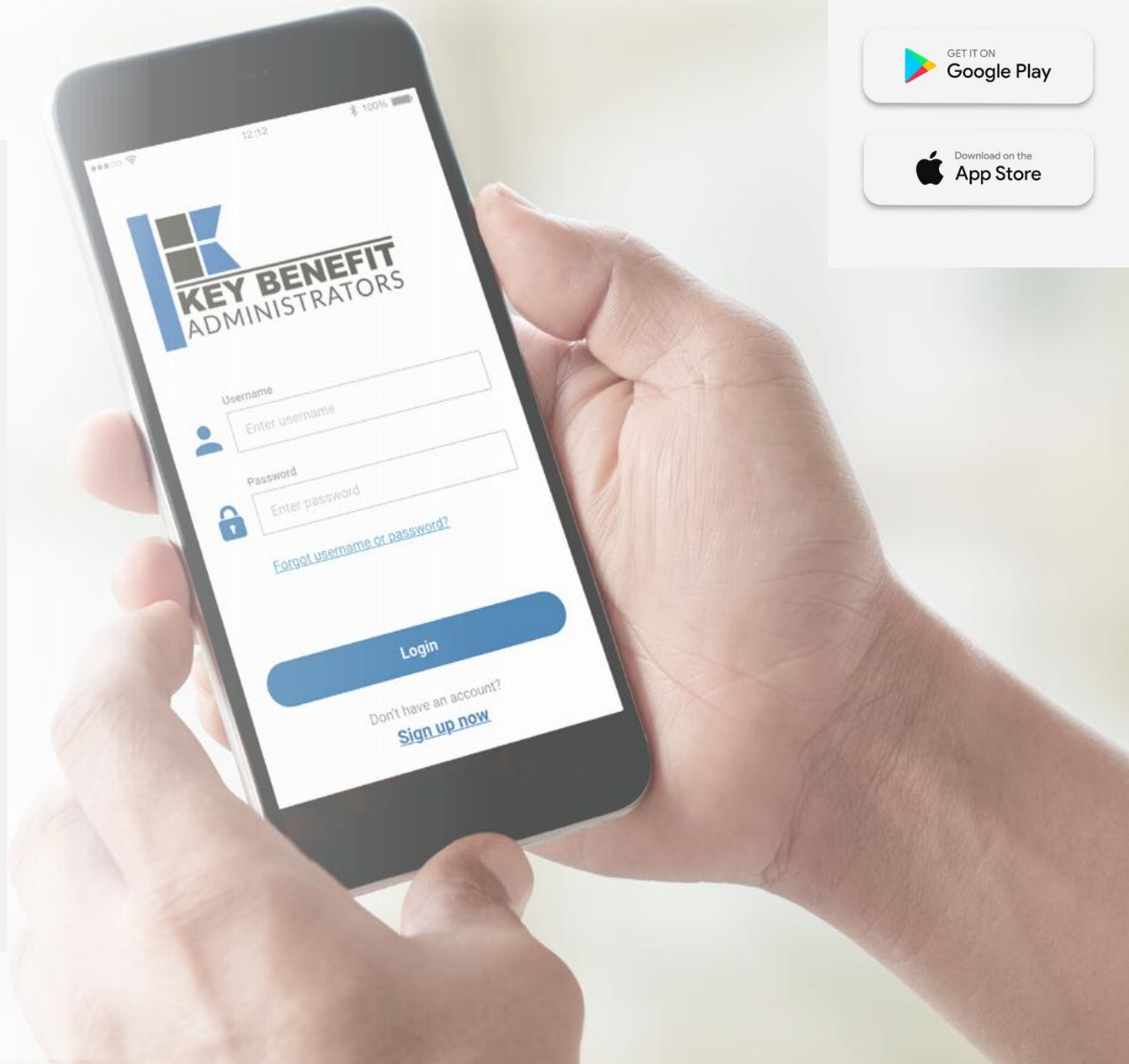
- EZView analytics platform provides instant access to critical analytics insights.
- Filter to a population by data source, group, plan, or geographic location.
- Identifications of chronic conditions, patient acuity, compliance with nurse coaching, regimens of care, steerage rates or claims costs.



Web Portal and Mobile Application

Gives you accessibility to your benefit plan information, claims data, eligibility details and much more!

- Biometric Login for ease of access (Fingerprint & Facial Recognition)
- Simplified Accumulator View
- Recent Claims Summary and Full Detail
- Benefit Information
- View/Request an ID Card (Display - Print – Fax – Email)
- SSO directly to Vendor Solutions
- Rx Price Comparison and Pharmacy Discount Programs (if applicable)
- Express Requests to Customer Support
- Push Notifications (Mobile App)



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Measuring Success: Customer Experience

Key Performance Indicators

Metric	2025 YTD	Industry Benchmark	How We Compare	
Abandonment Rate	1.04%	3–5% (acceptable), <2% (best-in-class)	Excellent 🏆	✓ Industry-leading responsiveness 1.04% abandonment and 17 s average speed of answer—minimal dropped calls, maximum access. ✓ Outstanding quality & access 90% of calls answered within 30s (vs. 80% target) and 99%+ audit scores across voice, email & chat. ✓ Balanced efficiency & depth Handle time reflects the detailed, compassionate TPA support your members need.
Average Speed of Answer (ASA)	17 seconds	<28 seconds (typical target); ~3 min in healthcare BPO	Excellent 🏆	
SLA (80% ≤ 30 sec)	90.00%	80% in 20–30 seconds (standard)	Above target ✅	
Average Handle Time	7 minutes 47 seconds	4–6 minutes typical; 7–8+ mins for complex TPA calls	Higher than industry, but expected	
After Call Work (ACW)	49 seconds	30–60 seconds typical	Within range 🟢	
Quality Audits (Email, Chat, Soft Skills, Procedural)	99%+	95–97% considered strong	Outstanding 100 🏆	

KBA DEDICATED TEAM ROLES

Our expert team is committed to delivering tailored solutions and exceptional service, ensuring consistent support and strategic guidance for every client.

Client Vice President (CVP)

Average Tenure: 15 Years

A dedicated individual delivering solutions tailored to client needs, driven by their unique data. The KBA CVP is deeply knowledgeable about client demographics, goals, and plans, as well as the health status of their membership. Through unparalleled analytics, the CVP illuminates the intelligence of the data, coordinates and conducts plan performance reviews, and provides meaningful recommendations. The CVP can also assist with employee enrollment meetings, presentations, and announcement materials, staying informed on client issues and initiatives, and tracking all account-related information.

Benefit Consultant (BC)

Average Tenure: 5 Years

Delivers strategic consultation to the client in conjunction with the CVP, with a primary focus on reporting and strategic insights. During renewal season, the BC assists in preparing comprehensive renewal proposals that include detailed service offerings, clear fee structures, and defined timelines. They also coordinate and conduct employee enrollment meetings as needed, ensuring a seamless and informed process for all stakeholders.

Account Representative (AR)

Average Tenure: 3 Years

Manages the day-to-day needs of the account, ensuring consistent communications, timely follow-up services, and continuous account monitoring. The AR oversees new case implementation and renewal processes alongside the KBA operations team. Responsibilities include the distribution of monthly reports, facilitating plan design changes, and obtaining signatures for plan documents and amendments. Additionally, they assist with funding management, including ACH transfer of funds and various other inquiries to ensure smooth financial operations.



ADDITIONAL KBA TEAM MEMBERS

On-Site KBA Support

An on-site resource for members to ask questions regarding benefits, eligibility, or claims. This role also provides essential HR support for support for plan administration, ensuring smooth operations and direct assistance.

Economist/Analyst

George Avery, PhD, MPA: Dr. Avery holds a doctorate in Health Services Research, Policy, and Administration from the University of Minnesota. With extensive academic experience as a professor at Purdue University and the University of Minnesota Duluth, he has contributed significantly to his field. His impressive publication record includes six book chapters and over 20 peer-reviewed journal articles.

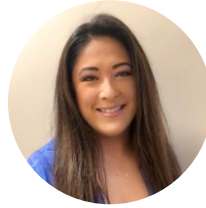
Dr. Avery's expertise is highly sought after, evidenced by his advisory roles for prestigious organizations such as the Indiana Office of Rural Health, the Centers for Disease Control and Prevention, the Department of Defense, and the United States Environmental Protection Agency. He brings over 30 years of invaluable experience in public health, health economics, and econometric analysis to the team.



DTE Implementation Team

The DTE Implementation Team is responsible for the overall management of the DTE project plan for the DTE Network and KBA Administrative Operations.

We are always excited about new partnerships and look forward to developing a solid foundation to build upon.



Christine Vinesett

Director, Implementation and Network

I joined Key Benefit Administrators in 2000 and have been in the Health Insurance Industry for 20+ years. My focus has been Leadership in Operations, Carrier and Provider Network Relations and Implementation Management.

Office: (803) 323-7974 | Email: Christine.Vinesett@keybenefit.com



Julie Caskey

Network Operations Lead

Julie Caskey joined Key Benefit Administrators (KBA) in 2021. Julie brings an extensive background with 30+ years in the Health Insurance Industry. Julie's focus has been in Provider Network and Relationship Management.

Office: 317-712-4003 | Email: Julie.Caskey@keybenefit.com



Dejanetta (Deja)
Pettaway

DTE Implementation and Relationship Coordinator

Deja joined Key Benefit Administrators in 2023 and has been in the Health Insurance Industry for 10 years. Deja's focus has been in Provider Network and Relationship Management.

Office: 803-396-4836 | Email: Dejanetta.Pettaway@keybenefit.com

DTE IMPLEMENTATION OVERVIEW

Phase 1	Phase 2		Phase 3	
DTE Network Setup	DTE Marketing & Website Setup	DTE Provider File Setup	Group Sales & Plan Design Setup	Group Administration Setup
☐ Legal Contracts and Agreements	☐ DTE Name Established	☐ Provider data and file analysis	☐ Group Sales and Marketing Strategy	☐ Proposal system setup
☐ Security and Compliance review	☐ DTE logo and messaging	☐ Provider file requirements	☐ Group Sales collateral	☐ Group Implementation and Renewal Collateral
☐ Secure email connection - Transport Layer Security (TLS)	☐ DTE Marketing Collateral	☐ Provider file transfer	☐ Group plan design and benefit structure	☐ Administration system programming (DTE product, plan design, billing, distribution, extract files)
☐ Secure file and reporting connection - Secure File Transfer Protocol (SFTP)	☐ DTE Website URL (domain)	☐ Provider file programming and website testing	☐ PBM plans and benefits review	☐ Claims adjudication system programming
☐ Network review of providers, specialties and sub-specialties	☐ DTE Website design and content creation	☐ Provider maintenance file established	☐ PBM structure analysis (systems, data import, data export, billing and invoicing)	☐ Fulfillment programming
☐ Network contracting and pricing analysis	☐ DTE Website provider portal setup	☐ DTE Website review for launch	☐ PBM file structure programming and testing (claims, accumulators, elig, invoice feed)	☐ Customer Service programming
☐ Network fee schedule programming and testing			☐ Reinsurance Carrier education on DTE	☐ Complete End to End Testing
Network launch	Website launch		Group Sales launch	

Group Implementation and Client Relations Team

Group implementations are assigned members from the Implementation and Client Relations Team. KBA is committed to delivering a “WOW” level of customer service and our team will ensure a smooth and timely transition to KBA.

It is our priority to execute a successful implementation.

Group Implementation Leadership

- Implementation Director
 - Christine Vinesett
 - Email: Christine.Vinesett@keybenefit.com | Phone: (803) 323-7974
- Implementation Manager
 - Rachel Morris
 - Email: Rachel.morris@keybenefit.com | Phone: (803) 396-4775
- Implementation Lead
 - Ashley Gowen
 - Email: Ashley.gowen@keybenefit.com | Phone: (317) 284-7863
- ❖ Implementation Coordinator's will be assigned at kick-off.

Client Relations Leadership

- Senior Vice President, Client Relations
 - Nicole Oliver
 - Email: Nicole.oliver@keybenefit.com | Phone: (803) 396-4720
- ❖ Client Relations VP and Account Representative will be assigned at kick-off.

GROUP IMPLEMENTATION OVERVIEW

Phase 1	Phase 2	Phase 3	Phase 4
Implementation Begins	Group Details - Open Enrollment, Eligibility Setup, ID Card, Banking, and Vendors	Finalize Setups - Open Enrollment, Eligibility, ID Card, Banking and Vendors	Confirmations - Fulfillment, Post Implementation Planning, GO LIVE!
☐ Identify key group contacts and their Roles and Responsibilities.	☐ Confirm Open Enrollment dates.	☐ Introduce Billing and Customer Service Teams.	☐ Confirm eligibility loaded.
☐ Review contracts and electronic signature process.	☐ Discuss materials needed for Open Enrollment and expected delivery.	☐ Process Open Enrollment eligibility.	☐ Confirm fulfillment mail date.
☐ Confirm Signor information.	☐ Discuss COBRA Administration. (if applicable)	☐ Finalize COBRA Administration setup. (if applicable)	☐ Confirm COBRA letter and packet mail dates. (if applicable)
☐ Establish meeting schedule, frequency and set target dates.	☐ Discuss Customer Service setup.	☐ Finalize EZ Benefits setup.	☐ Confirm all group setups are complete.
☐ Begin KBA Client Profile.	☐ Review EDI Setup and initial eligibility file submission.	☐ Finalize ID Card setup.	☐ Confirm plan document has been signed.
☐ Request prior plan document.	☐ Schedule EDI meeting. (if applicable)	☐ Finalize banking setups.	☐ Deliver 1st Months Invoice and finalize.
☐ Review plan document, benefits and discuss changes.	☐ Review EZ Benefits platform.	☐ Finalize claims and EOB setups.	☐ Deliver final reinsurance contract.
☐ Request claim file and accumulator data.	☐ Discuss ID Card setup, approval process and delivery expectations.	☐ Finalize PBM Implementation.	☐ Discuss reporting needs for plan year.
☐ Review Fee Schedule, Implementation Fee.	☐ Discuss Banking and Funding setup for monthly premiums and claims.	☐ Finalize Plan Document.	☐ Complete full transition to Client Relationship Team for administration.
☐ Discuss Notice of Termination to prior carrier and obtain copy.	☐ Discuss frequency of check runs, large dollar claims review, exception process and EOBs.	☐ Review status of EDI setup. (if applicable)	
	☐ Discuss monthly premium billing preferences.		
	☐ Begin PBM Implementation and schedule meetings.		
	☐ Discuss vendor setup process.		
	☐ Review special programs. (if		

COLLABORATION OF STAKEHOLDERS



Key Benefit Administrators

- Full TPA Administration
- Stop-Loss Administration
- True Transparency & Analytics
- Population Health Management
- Utilization & Case Management
- Sales & Marketing
- Transparent PBM
- Website & Mobile App
- Customizable options & plans



MU Healthcare's DTE

- Incremental Revenues and Margin
- Branding & Messaging
- Benefit Plan Integration
- Network Steerage
- Direct Relationship with Self-Funded Employers
- Referral Authorization
- Plan Transparency
- Direct Access to Members



Employer

- Reduced Spend
- "At The Table" with MU Healthcare
- Improved Analytics
- Complete Transparency
- Lower Stop-Loss
- Benefit Plan Stabilization
- Healthier Population



Employee

- Reduced Out of Pocket
- Improved Health
- Better Coordination & Navigation of Care
- Direct Access to the Best Medical Providers

ROLES & RESPONSIBILITIES

SERVICES & ADMINISTRATIVE SYSTEMS PROVIDED BY:	
MU Healthcare	KBA Operations
▪ Tier 1 Provider Network ▪ Provider Recruitment – If Necessary ▪ Network Pricing & Contract Terms ▪ Referral Management Protocols & Approvals ▪ Medical Director Oversight ▪ Practice Improvement Program using KBA/AHDI Data with Assistance from KBA	▪ Network Evaluation – Actuarial Pricing ▪ PPO Wrap & Tertiary Transplant Network, if needed ▪ Claims Administration ▪ Plan Design Development, Testing, and Administration ▪ Employer/Member/Provider Customer Service ▪ Customized Member/Provider/Agent/HR Portal ▪ Billing, Enrollment (Electronic/Paper), and Premium Management ▪ Fulfilment – SPD & Customized ID Cards ▪ COBRA/HIPAA Administration ▪ Out of Network Negotiations ▪ Wrap PPO Repricing ▪ Nurse Coaching for Chronically Ill ▪ Nurse Triage to Physicians and Facilities ▪ Stop-Loss Underwriting, Pricing, and Placement ▪ Stop Loss Claims Administration ▪ Complex Case Management ▪ PBM Administration Interface ▪ Reporting • Utilization & Gaps in Care • Clinically Based Physician Profiling • Referrals • Chronic Disease Management ▪ Employer Health Risk Management Reports ▪ Ancillary Administration – Dental, Vision, Weekly Income, 5500, Life, etc. ▪ Health Savings Account (HSA), Flexible Spending (FSA), Health Reimbursement Account (HRA) Administration ▪ Sales & Marketing Plan • Strategy Development: Branding, Brochures, Web Site • Distribution System Development • Sales Management
KBA Consulting	
▪ Program oversight and integration across the enterprise ▪ Goal development, agreement and tracking ▪ Start-up assessments: market, network, operational and readiness ▪ Draft business plan with financial projections ▪ Facilitate program communication and engagement with MU Healthcare departments including system c-suite, operations, HR, managed care, marketing, strategy and business development	

An aerial photograph of a city skyline under a blue sky with wispy clouds. A semi-transparent white rectangular box is centered over the middle of the image. The text "NETWORK MANAGEMENT" is written in large, bold, black capital letters across this box. In the background, various city buildings are visible, including a prominent brick building with a "TIGER" sign on its roof and a white building with a dome. In the foreground, a street intersection with traffic lights and cars is visible.

NETWORK MANAGEMENT

DTE Network Management Team

The DTE Contracting Team is responsible for DTE plan network management.

We work directly with our DTE partners to ensure all networks have the access needed for successful sales.



Marlin Wilcox

Vice President of Network Contracting

Marlin Wilcox is a longtime healthcare executive and entrepreneur with nearly four decades of experience building and leading health plans and provider networks. He began his career in managed care in the late 1980s as Director of Provider Network for Ohio Health Choice Plan in Cleveland, before serving as President of Primary Health Services and later DirectCare America in Aurora, Ohio. In 1996, he became President of Opn, where he led the organization for nearly 15 years. Since 2012, Wilcox has served as Owner, President, and CEO of Health Ohio Network, reflecting his sustained focus on developing alternative health plan models and network solutions.

Email: marlin.wilcox@keybenefit.com



Jamie Field

Provider Relations DTE Leader

Jamie Field is a seasoned healthcare insurance and provider network professional with more than 30 years of experience spanning sales, account management, claims operations, and network development. She currently serves as a Provider Relations Representative at Key Benefit, where she supports provider engagement and network growth. Previously, she was a Sales Specialist at UnitedHealthcare and a licensed health insurance agent in seventeen states, specializing in Medicare and Part D. Throughout her career—including leadership roles at hChoices, University Hospitals of Cleveland Corporate Health, PPOM/Flora Medwest, HealthStar, and The Chandler Group—Jamie has built and managed large provider networks, driven consistent double-digit business growth, implemented new clients and operating systems, developed training programs, and strengthened multi-level client relationships.

Email: jamie.field@keybenefit.com

DTE Analytics Leader

Taylor Davis is a healthcare executive with a Master's degree in Statistics and deep experience at the intersection of data, strategy, and executive leadership. As President of KLAS, he led one of the most respected healthcare research and advisory organizations in the industry, working closely with health system and vendor executives nationwide to drive performance improvement and transparency. With a strong analytical foundation and extensive boardroom experience, Taylor brings a rare combination of quantitative rigor and strategic insight to healthcare innovation, market evaluation, and organizational growth.

Email: taylor.davis@keybenefit.com



Taylor Davis

The Healthcare Provider Landscape Challenge

The Columbia Missouri healthcare market presents a complex web of care providers, specialty providers, specialty groups, and healthcare facilities that can overwhelm patients, employers, employers, and payers alike.

Market Complexity

- Hundreds of independent provider provider groups
- Multiple health system affiliations
- Varying quality and pricing structures structures
- Limited transparency in outcomes

The Challenge

Without strategic curation and management, this overwhelming number of number of care groups creates confusion, confusion, inefficiencies, and suboptimal suboptimal outcomes for all stakeholders. stakeholders.

The solution: A carefully managed, high-value network.



Our Goal in Partnering With MU Health Care



Build the Highest-Value Network

Create the Dallas-Fort Worth area's premier healthcare network that delivers exceptional quality care with market-leading pricing, setting the gold standard for value-based healthcare delivery.



Enable Fastest-Possible Growth

Leverage our proven methodologies and established MU Health Care relationships to accelerate network expansion, ensuring rapid market penetration and comprehensive coverage across the DFW metropolitan area.



Vault MU Health Care to Competitive Advantage

Position MU Health Care as the anchor and leader of this transformative network, creating sustainable competitive advantages that drive market leadership and long-term success.

"Value is high-quality care at a great price – this principle guides every decision we make in network development and provider selection."



HEALTHCARE
PARTNERSHIP

Networks Must Be Carefully Tended

Proven Network Leadership

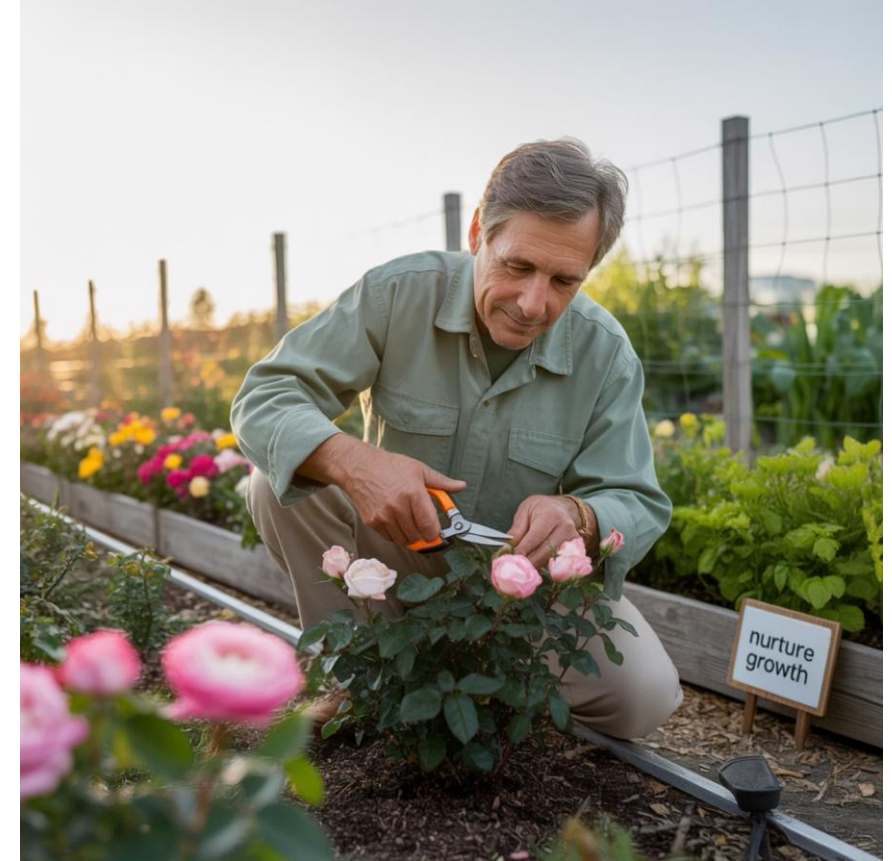
Our leadership team brings unparalleled depth of experience to healthcare network management and strategic development.

Each team member has over 25 years of hands-on experience in healthcare network operations, operations, provider relations, strategic planning and contracting.

Our Management Approach

- **Network integrity monitoring** – maintaining quality and access standards
- **Provider relations management** – fostering strong, productive partnerships
- **Performance analytics** – continuous improvement through data-driven insights

We manage all the tending always in cooperation with the sponsoring provider organization.
organization.



Vehicle for Approving Network Changes: Touch Point Meetings

Regular touchpoint meetings serve as the strategic command center for network development, ensuring every addition aligns with organizational goals and market needs.

01

Network Analysis

Comprehensive evaluation of current network gaps, utilization patterns, and performance metrics to identify strategic opportunities.

02

Stakeholder Feedback

Regular collection and analysis of feedback from employers, brokers, and members to ensure network meets real-world needs and expectations.

03

Option Review

Systematic evaluation of potential provider additions, considering clinical quality, geographic geographic coverage, and financial impact.

04

Strategic Recommendations

Data-driven recommendations presented to leadership for informed decision-making on network expansion and optimization.

05

Contracting Execution

Upon agreement, we approach and negotiate with target provider groups to groups to secure favorable terms and seamless integration.



Who Needs to Be in the Touchpoints

i Critical Success Factor: Dedicated system leaders who can make real-time decisions about network inclusion and exclusion criteria.

Decision-Making Authority

Touchpoint meetings require participants to advise network changes, contract terms (where terms (where possible), and strategic direction.

Meeting Structure & Frequency

- **Weekly meetings** for the first several months during initial network build-out
- **Monthly meetings** once network reaches maturity
- **As-needed sessions** for urgent opportunities or issues

Required Participants

- Network leadership with contracting authority
- Clinical quality decision-makers
- Financial/pricing stakeholders
- Strategic planning representatives

The group collectively decides the meeting structure and governance approach that works best for their organization.

Contract Negotiations & Risk Management



Comprehensive Negotiation Management

We handle all contract language, terms, and subsequent subsequent reimbursement negotiations, ensuring optimal optimal terms while protecting your organization from legal from legal and financial risks.



Price-Fixing Protection

By managing all rate negotiations directly, we create a protective barrier that keeps MU Health Care safe from potential price-fixing concerns and antitrust issues.



Annual Renewal Excellence

All negotiations are renewed yearly, and our strong track record of delivering excellent reimbursements to providers enables smoother renewals and stronger relationships.

Our Negotiation Advantages

Our extensive experience across multiple markets gives us unique insights into fair market rates and industry best practices. We leverage practices. We leverage data analytics and market intelligence to secure favorable terms for both the network and participating providers. participating providers.





WE REMEMBER WHO IS GIVING THE CARE

A PARTNER YOU CAN TRUST



**OUR OBJECTIVES
ARE ALIGNED**



**WE KNOW THE
MARKET**



**COMPLETELY
TRANSPARENT**



**WE HAVE THE
SOPHISTICATION
AND
INFRASTRUCTURE
TO EFFECTIVELY
COMPETE**



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